

PARTICIPATING PROVIDER AGREEMENT

This Participating Provider Agreement (together with all Attachments and amendments, this “Agreement”) is made and entered by and between RadiantCare Physicians PLLC (“Provider”), Coordinated Care Corporation, a health maintenance organization (“CCC”) and Coordinated Care of Washington, Inc. (“CCW”) (each a “Party” and collectively the “Parties”). This Agreement is effective as of the date designated by Health Plan (as defined herein) on the signature page of this Agreement (“Effective Date”).

WHEREAS, Provider desires to provide certain health care services to individuals in products offered by or available from or through a Company or Payor (as hereafter defined), and Provider desires to participate in such products as a Participating Provider (as defined herein), all as hereinafter set forth.

WHEREAS, Health Plan (as defined herein) desires for Provider to provide such health care services to individuals in such products, and Health Plan desires to have Provider participate in certain of such products as a Participating Provider, all as hereinafter set forth.

NOW, THEREFORE, in consideration of the recitals and mutual promises herein stated, the Parties hereby agree to the provisions set forth below.

ARTICLE I - DEFINITIONS

When appearing with initial capital letters in this Agreement (including an Attachment), the following quoted and underlined terms (and the plural thereof, when appropriate) have the meanings set forth below.

1.1. “Affiliate” means a person or entity controlling, controlled by, or under common control with Health Plan.

1.2. “Attachment” means any document, including an addendum, schedule or exhibit, attached to this Agreement as of the Effective Date or that becomes attached pursuant to Section 2.2 or Section 8.7, all of which are incorporated herein by reference and may be amended from time to time as provided in this Agreement.

1.3. “Clean Claim” means a claim for payment that has no defect or impropriety, including any lack of any required substantiating documentation, or particular circumstances requiring special treatment that prevents timely payments from being made on the claim.

1.4. “Company” means (collectively or individually, as appropriate in the context) Health Plan and its Affiliates, except those specifically excluded by Health Plan.

1.5. “Compensation Schedule” means at any given time the then effective schedule(s) of maximum rates applicable to a particular Product under which Provider and Contracted Providers will be compensated for the provision of Covered Services to Covered Persons. Such Compensation Schedule(s) will be set forth or described in one or more Attachments to this Agreement, and may be included within a Product Attachment.

1.6. “Contracted Provider” means a physician, hospital, health care professional or any other provider of items or services that is employed by or has a contractual relationship with Provider. The term “Contracted Provider” includes Provider for those Covered Services provided by Provider.

1.7. “Coverage Agreement” means any agreement, program or certificate entered into, issued or agreed to by Company or Payor, under which Company or Payor furnishes administrative services or other services in support of a health care program for an individual or group of individuals, and which may include access to one or more of Company’s provider networks or vendor arrangements, except those excluded by Health Plan.

1.8. “Covered Person” means any individual entitled to receive Covered Services pursuant to the terms of a Coverage Agreement.

1.9. “Covered Services” means those services and items for which benefits are available and payable under the applicable Coverage Agreement and which are determined, if applicable, to be Medically Necessary under the applicable Coverage Agreement.

1.10. “Health Plan” means either CCC or CCW with respect to each Product covered by this Agreement, as specified in accordance with Schedule B to this Agreement or the applicable Product Attachment.

1.11. “Medically Necessary” or “Medical Necessity” shall have the meaning defined in the applicable Coverage Agreement and applicable Regulatory Requirements.

1.12. “Participating Provider” means, with respect to a particular Product, any physician, hospital, ancillary, or other health care provider that has contracted, directly or indirectly, with Health Plan to provide Covered Services to Covered Persons, that has been approved for participation by Company, and that is designated by Company as a “participating provider” in such Product.

1.13. “Payor” means the entity (including Company where applicable) that bears direct financial responsibility for paying from its own funds, without reimbursement from another entity, the cost of Covered Services rendered to Covered Persons under a Coverage Agreement and, if such entity is not Company, such entity contracts, directly or indirectly, with Company for the provision of certain administrative or other services with respect to such Coverage Agreement.

1.14. “Payor Contract” means the contract with a Payor, pursuant to which Company furnishes administrative services or other services in support of the Coverage Agreements entered into, issued or agreed to by a Payor, which services may include access to one or more of Company’s provider networks or vendor arrangements, except those excluded by Health Plan. The term “Payor Contract” includes Company’s or other Payor’s contract with a governmental authority (also referred to herein as a “Governmental Contract”) under which Company or Payor arranges for the provision of Covered Services to Covered Persons.

1.15. “Product” means any program or health benefit arrangement designated as a “product” by Health Plan (e.g., Health Plan Product, Medicaid Product, PPO Product, Payor-specific Product, etc.) that is now or hereafter offered by or available from or through Company (and includes the Coverage Agreements that access, or are issued or entered into in connection with such product, except those excluded by Health Plan).

1.16. “Product Attachment” means an Attachment setting forth requirements, terms and conditions specific or applicable to one or more Products, including certain provisions that must be included in a provider agreement under the Regulatory Requirements, which may be alternatives to, or in addition to, the requirements, terms and conditions set forth in this Agreement.

1.17. “Regulatory Requirements” means all applicable federal and state statutes, regulations, regulatory guidance, judicial or administrative rulings, requirements of Governmental Contracts and standards and requirements of any accrediting or certifying organization, including, but not limited to, the requirements set forth in a Product Attachment.

1.18. “State” is defined as the State of Washington.

ARTICLE II – PRODUCTS AND SERVICES

2.1. Contracted Providers. Provider shall, and shall cause each Contracted Provider, to comply with and abide by the agreements, representations, warranties, acknowledgements, certifications, terms and conditions of this Agreement (including the provisions of Schedule A that are applicable to Provider, a Contracted Provider, or

their services, and any other Attachments) and fulfill all of the duties, responsibilities and obligations imposed on Provider and Contracted Providers under this Agreement (including each Attachment).

2.2. Products and Attachments. Subject to the other provisions of this Agreement, Provider and each Contracted Provider is subject to and bound by all Attachments designated on Schedule B of this Agreement, and may be identified as a Participating Provider in each Product identified in a Product Attachment designated on Schedule B of this Agreement.

2.2.1. Provider shall, at all times during the term of this Agreement, require each of its Contracted Providers to, subject to Company's approval, participate as Participating Providers in each Product identified in a Product Attachment that is designated on Schedule B of this Agreement or added to this Agreement in accordance with Section 2.2 hereof.

2.2.2. A Contracted Provider may only identify itself as a Participating Provider for those Products in which the Contracted Provider actually participates as provided in this Agreement. Provider acknowledges that Company or Payor may have, develop or contract to develop various Products or provider networks that have a variety of provider panels, program components and other requirements. No Company or Payor warrants or guarantees that any Contracted Provider: (i) will participate in all or a minimum number of provider panels, (ii) will be utilized by a minimum number of Covered Persons, or (iii) will indefinitely remain a Participating Provider or member of the provider panel for a particular network or Product.

2.2.3. Attached hereto as Schedule C is the initial list of the Contracted Providers as of the Effective Date. Provider shall provide Health Plan on a quarterly basis or more often upon request with a complete and accurate list containing the names, office telephone numbers, addresses, tax identification numbers, hospital affiliations, specialties and board status (if applicable), languages spoken, whether Contracted Provider is accepting new patients, State license number, and National Provider Identifier of Contracted Providers and such other information as required to remain in compliance with all applicable Regulatory Requirements, and shall provide Health Plan with a list of modifications to such list at least thirty (30) days prior to the effective date of such changes. Provider shall provide such lists in a manner and format mutually acceptable to the Parties.

2.2.4. Provider may add new providers to this Agreement as Contracted Providers. In such case, Provider shall provide written notice to Health Plan of the prospective addition(s) and shall use best efforts to provide such notice at least sixty (60) days in advance of such addition. Provider shall maintain written agreements with each of its Contracted Providers (other than Provider) that require Contracted Providers to comply with the terms and conditions of this Agreement and that address and comply with the Regulatory Requirements.

2.2.5. If Company desires to add an additional Product, Company or Payor, as applicable, will provide at least sixty (60) days' prior written notice (electronic or paper) thereof to Provider, along with the applicable Product Attachment and the new Compensation Schedule, if any. Contracted Providers will not be designated as Participating Providers in such additional Product until Provider agrees to participate in such additional Product by giving Company or Payor, as applicable, written notice of its decision to participate in accordance with the process specified in the notice to Provider. If Provider gives timely notice of agreement to participate in an additional Product, then each Contracted Provider shall be a Participating Provider in such additional Product on the terms and conditions set forth in this Agreement and the applicable Product Attachment.

2.3. Covered Services. Each Contracted Provider shall provide Covered Services described or referenced in the applicable Product Attachment(s) to Covered Persons in those Products in which the Contracted Provider is a Participating Provider in accordance with this Agreement. Each Contracted Provider shall provide Covered Services to Covered Persons with the same degree of care and skill as customarily provided to patients who are not Covered Persons, within the scope of the Contracted Provider's license and in accordance with generally accepted standards of the Contracted Provider's practice and business and in accordance with the provisions of this Agreement and Regulatory Requirements.

2.4. Policies and Procedures. Provider warrants that Provider and Contracted Providers shall at all times cooperate and comply with applicable administrative requirements, policies, programs and procedures of Company and Payor, which may include, but are not limited to, the following: credentialing criteria and requirements; confidentiality and notification requirements; medical management programs; claims and billing, quality assessment and improvement, utilization review and management, disease management, case management, on-site reviews, referral and prior authorization, and grievance and appeal procedures; coordination of benefits and third party liability policies; carve-out and third party vendor programs; and data reporting requirements. The failure to comply with such policies and procedures could result in a denial or reduction of payment to the Provider or Contracted Provider or a denial or reduction of the Covered Person's benefits. Such policies and procedures do not in any way affect or remove the obligation of Contracted Providers to render care. Health Plan shall make the applicable policies available to Provider and Contracted Providers prior to contracting and throughout the term of the Agreement upon reasonable request via one or more designated websites or alternative means. Company shall notify Provider at least sixty (60) days in advance of changes in administrative policies and procedures that affect Provider's compensation or health care service delivery unless changes to federal or State law or regulations make such advance notice impossible, in which case notice will be provided as soon as possible. Such notice may be given by Health Plan through an update to information available to Provider online, or any other written method (electronic or paper). Provider shall notify Contracted Providers of such changes.

2.5. Credentialing Criteria. Provider and each Contracted Provider shall complete Company's and/or Payor's credentialing and/or recredentialing process as required by Company's and/or Payor's credentialing policies, and shall at all times during the term of this Agreement meet all of Company's and/or Payor's credentialing criteria. Provider and each Contracted Provider represents, warrants and agrees: (a) that it is currently, and for the duration of this Agreement shall remain: (i) in compliance with all applicable Regulatory Requirements, including licensing laws; (ii) if applicable, accredited by The Joint Commission or the American Osteopathic Association; and (iii) a Medicare-certified provider under the federal Medicare program and a Medicaid participating provider under applicable federal and State laws; and (b) that all Contracted Providers and all employees and contractors thereof will perform their duties in accordance with all Regulatory Requirements, as well as applicable national, State and local standards of professional ethics and practice. No Contracted Provider shall provide Covered Services to Covered Persons or identify itself as a Participating Provider unless and until the Contracted Provider has been notified, in writing, by Company that such Contracted Provider has successfully completed Company's credentialing process.

2.6. Eligibility Determinations. Provider or Contracted Provider shall timely verify whether an individual seeking Covered Services is a Covered Person. Company or Payor, as applicable, will make available to Provider and Contracted Providers a method whereby Provider and Contracted Providers can obtain, in a timely manner, general information about eligibility and coverage. Company or Payor, as applicable, does not guarantee that persons identified as Covered Persons are eligible for benefits or that all services or supplies are Covered Services. If Company, Payor or its delegate determines that an individual was not a Covered Person at the time services were rendered, such services shall not be eligible for payment under this Agreement, except to the extent such services were expressly authorized by Company or Payor. For retrospective review, eligibility determinations will be made solely on the medical information available to the Contracted Provider at the time the health service was provided. Such retrospective review will be completed within thirty (30) calendar days of receipt of the necessary information. In addition, Company will use reasonable efforts to include or contractually require Payors to clearly display Company's name, logo or mailing address (or other identifier(s) designated from time to time by Company) on each membership card.

2.7. Referral and Preauthorization Procedures. Provider and Contracted Providers shall comply with referral and preauthorization procedures adopted by Company and/or Payor, as applicable, prior to referring a Covered Person to any individual, institutional or ancillary health care provider. Unless otherwise expressly authorized in writing by Company or Payor, Provider and Contracted Providers shall refer Covered Persons only to Participating Providers to provide the Covered Service for which the Covered Person is referred. Except as required by applicable law, failure of Provider and Contracted Providers to follow such procedures may result in denial of payment for unauthorized treatment. Preauthorization is not required prior to provision of Covered Services in the event of an emergency.

2.7.1 Prior Authorization: In accordance with RCW 48.43.16(2)(a), Provider Agreement does not require utilization management or review of any kind for an initial evaluation and management visit, and up to six treatment visits with a contracting provider in a new episode of care for each of the following:

- Chiropractic
- Physical therapy
- Occupational therapy
- Acupuncture and Eastern medicine
- Massage therapy
- Speech therapy

Visits where utilization management or review is prohibited are still subject to quantitative treatment limits of the Health Plan. With the exception of RCW 48.43.515(5), the Health Plan can require a referral or prescription for the therapists listed.

For visits where utilization management or review is prohibited, Health Plan will not deny or limit coverage on the basis of medical necessity or appropriateness; or retroactively deny care or refuse payment for the visits in accordance with RCW 48.43.016(2)(b).

2.8. Treatment Decisions. No Company or Payor is liable for, nor will it exercise control over, the manner or method by which a Contracted Provider provides items or services under this Agreement. Provider and Contracted Providers understand that determinations of Company or Payor that certain items or services are not Covered Services or have not been provided or billed in accordance with the requirements of this Agreement are administrative decisions only. Such decisions do not absolve the Contracted Provider of its responsibility to exercise independent judgment in treatment decisions relating to Covered Persons. Nothing in this Agreement (i) is intended to interfere with Contracted Provider's relationship with Covered Persons, or (ii) prohibits or restricts a Contracted Provider from disclosing to any Covered Person any information that the Contracted Provider deems appropriate regarding health care quality, medical treatment decisions or alternatives.

2.8.1 Withdrawal Management Services: In accordance with RCW 48.43.761 and WAC 284-43-2000, Provider Agreement may not specify timeframes for substance abuse disorder treatment less than what is specified in RCW 48.43.761(12)(a)(i), nor specify timeframes for withdrawal management services less than what is specified in RCW 48.43.761(2)(a)(ii), nor contain language that prevents a seamless transfer to an appropriate level of care.

2.9. Carve-Out Vendors. Provider acknowledges that Company may, during the term of this Agreement, carve-out certain Covered Services from its general provider contracts, including this Agreement, for one or more Products as Company deems necessary or appropriate. Provider and Contracted Providers shall cooperate with and, when medically appropriate, utilize all third party vendors designated by Company for those Covered Services identified by Company from time to time for a particular Product.

2.10. Disparagement Prohibition. Provider, each Contracted Provider and the officers of Company shall not disparage the other during the term of this Agreement or in connection with any expiration, termination or non-renewal of this Agreement. Neither Provider nor Contracted Provider shall interfere with Company's direct or indirect contractual relationships including, but not limited to, those with Covered Persons or other Participating Providers. Nothing in this Agreement should be construed as limiting the ability of Health Plan, Company, Provider or a Contracted Provider to inform Covered Persons that this Agreement has been terminated or otherwise expired or, with respect to Provider, to promote Provider to the general public, or to limit Contracted Providers from participating in discussions with a patient or someone paying for their coverage regarding the comparative merits of different health carriers, even if critical of a carrier, or to post information regarding other health plans consistent with Provider's usual procedures, provided that no such promotion or advertisement is specifically directed at one or more Covered Persons. In addition, nothing in this provision should be construed as limiting Company's ability to use and disclose information and data obtained from or about Provider or Contracted Provider, including this Agreement, to the extent determined reasonably necessary or appropriate by Company in

connection with its efforts to comply with Regulatory Requirements and to communicate with regulatory authorities.

2.11. Nondiscrimination. Provider and each Contracted Provider will provide Covered Services to Covered Persons without discrimination on account of race, sex, sexual orientation, age, color, religion, national origin, place of residence, health status, type of Payor, source of payment (e.g., Medicaid generally or a State-specific health care program), physical or mental disability or veteran status, and will ensure that its facilities are accessible as required by Title III of the Americans With Disabilities Act of 1991. This requirement does not require a Contracted Provider to render services that are not appropriate for the provider to render due to limitations arising from lack of training, experience, skill, or licensing restrictions. Provider and Contracted Providers recognize that, as a governmental contractor, Company or Payor may be subject to various federal laws, executive orders and regulations regarding equal opportunity and affirmative action, which also may be applicable to subcontractors, and Provider and each Contracted Provider agree to comply with such requirements as described in any applicable Attachment.

2.12. Notice of Certain Events. Provider shall give written notice to Health Plan of: (i) any event of which notice must be given to a licensing or accreditation agency or board; (ii) any change in the status of Provider's or a Contracted Provider's license; (iii) termination, suspension, exclusion or voluntary withdrawal of Provider or a Contracted Provider from any state or federal health care program, including but not limited to Medicaid; or (iv) any settlements or judgments in connection with a lawsuit or claim filed or asserted against Provider or a Contracted Provider alleging professional malpractice involving a Covered Person. In any instance described in subsection (i)-(iii) above, Provider must notify Health Plan or Payor in writing within ten (10) days, and in any instance described in subsection (iv) above, Provider must notify Health Plan or Payor in writing within thirty (30) days, from the date it first obtains knowledge of the pending of the same.

2.13. Use of Name. Provider and each Contracted Provider hereby authorizes each Company or Payor to use their respective names, telephone numbers, addresses, specialties, certifications, hospital affiliations (if any), and other descriptive characteristics of their facilities, practices and services for the purpose of identifying the Contracted Providers as "Participating Providers" in the applicable Products. Provider and Contracted Providers may only use the name of the applicable Company or Payor for purposes of identifying the Products in which they participate, and may not use the registered trademark or service mark of Company or Payor without prior written consent.

2.14. Compliance with Regulatory Requirements. Provider, each Contracted Provider and Company agree to carry out their respective obligations under this Agreement in accordance with all applicable Regulatory Requirements, including, but not limited to, Chapter 284-43 of the Washington Administrative Code, the Health Insurance Portability and Accountability Act, the Health Information Technology for Economic and Clinical Health (HITECH) Act, and federal drug and alcohol confidentiality laws in 42 C.F.R. Part 2, each as amended, including any regulations promulgated thereunder. If, due to Provider's or Contracted Provider's noncompliance with applicable Regulatory Requirements or this Agreement, sanctions or penalties are imposed on Company, Company may, in its sole discretion, offset such amounts against any amounts due Provider or Contracted Providers from any Company or require Provider or the Contracted Provider to reimburse Company for such amounts. If Provider subcontracts any services under this Agreement, then Provider is responsible for ensuring that its written agreements with such subcontractors contain all applicable Regulatory Requirements and that its subcontractors comply with such requirements.

2.15. Program Integrity Required Disclosures. Provider agrees to furnish to Health Plan complete and accurate information necessary to permit Health Plan to comply with the collection of disclosures requirements specified in 42 C.F.R. Part 455 Subpart B or any other applicable State or federal requirements, within such time period as is necessary to permit Health Plan to comply with such requirements. Such requirements include but are not limited to: (i) 42 C.F.R. §455.105, relating to (a) the ownership of any subcontractor with whom Provider has had business transactions totaling more than \$25,000 during the 12-month period ending on the date of the request and (b) any significant business transaction between Provider and any wholly owned supplier or subcontractor during the five (5) year period ending on the date of the request; (ii) 42 C.F.R. §455.104, relating to individuals or

entities with an ownership or controlling interest in Provider; and (iii) 42 C.F.R. §455.106, relating to individuals with an ownership or controlling interest in Provider, or who are managing employees of Provider, who have been convicted of a crime

2.16. Drug Utilization Management Exceptions. Provider drug utilization management exceptions are available on our website: www.coordinatedcarehealth.com.

2.17. Mental/Behavioral Health Providers: In accordance with RCW 48.087(2) and RCW 48.087(3), nothing in this contract will prevent a mental/behavioral health practitioner and an enrollee from agreeing to have services provide at the Covered Person's expense. If a mental/behavioral health practitioner provides services to an enrollee during an appeal or adverse certification process, the practitioner must provide written notification to the enrollee that payment for services is the enrollee's responsibility, unless Health Plan elects to pay.

2.18. Protection of Individual Right to Privacy & Confidential Services: In accordance with RCW 48.43.505, Health Plan does not require protected individuals to obtain permission from the policyholder, subscriber, or another covered person to receive care or submit a claim if they have the right to consent to care. Health Plan recognizes the right of a protected individual or enrollee to exercise their rights regarding health information related to care they receive. Health Plan directs all communications regarding a protected individual's receipt of sensitive health services to the patient receiving care, or via postal mail, e-mail, or telephone number specified by the protected individual. Health information may not be disclosed to anyone other than the protected individual without their written, or recorded verbal consent.

A protected individual may request communications regarding the receipt of sensitive health care services be sent to another individual or provider for the purposes of appealing adverse benefit determinations. Health Plan will limit disclosure of any information about a protected individual who is the subject of the information and will direct communications directly to the protected individual, or via mail, e-mail or phone number specified by the protected individual upon request. Protected individuals are not required to waive any right to limit disclosure as a condition of eligibility or coverage. To protect patient confidentiality, Health Plan communications disclosing protected health information or relating to sensitive services shall be provided in the form and format requested by the patient receiving care.

ARTICLE III – CLAIMS SUBMISSION, PROCESSING, AND COMPENSATION

3.1. Claims or Encounter Data Submission. Contracted Providers shall submit to Payor or its delegate claims for payment for Covered Services rendered to Covered Persons. Contracted Provider shall submit encounter data to Payor or its delegate in a timely fashion, which must contain statistical and descriptive medical and patient data and identifying information. Payor or its delegate reserves the right to deny payment to the Contracted Provider if the Contracted Provider fails to submit claims for payment or encounter data in accordance with the applicable policies and procedures.

3.2. Compensation. The compensation for Covered Services provided to a Covered Person ("Compensation Amount") will be the appropriate amount under the applicable Compensation Schedule in effect on the date of service for the Product in which the Covered Person participates. Subject to the terms of this Agreement, Provider and Contracted Providers shall accept the Compensation Amount as payment in full for the provision of Covered Services. Subject to the terms of this Agreement, Payor shall pay or arrange for payment of each Clean Claim received from a Contracted Provider for Covered Services provided to a Covered Person in accordance with the applicable Compensation Amount less any applicable copayments, cost-sharing or other amounts that are the Covered Person's financial responsibility under the applicable Coverage Agreement.

3.3. Financial Incentives. The Parties acknowledge and agree that nothing in this Agreement shall be construed to create any financial incentive for Provider or a Contracted Provider to withhold Covered Services.

3.4. Hold Harmless.

3.4.1 Provider and each Contracted Provider agree that in no event, including but not limited to non-payment by a Payor, a Payor's insolvency, or breach of this Agreement, shall Provider or a Contracted Provider bill, charge, collect a deposit from, seek compensation, remuneration or reimbursement from, or have any recourse against a Covered Person or person acting on the Covered Person's behalf, other than Payor, for Covered Services provided under this Agreement. This provision shall not prohibit collection of any applicable copayments, cost-sharing or other amounts, which have not otherwise been paid by a primary or secondary carrier in accordance with regulatory standards for coordination of benefits, that are the Covered Person's financial responsibility under the applicable Coverage Agreement.

3.4.2 Provider and each Contracted Provider agree, in the event of Payor's insolvency, to continue to provide the services promised in the Coverage Agreement to Covered Persons for the duration of the period for which premiums on behalf of the Covered Persons were paid to Company or Payor or until the Covered Person's discharge from inpatient facilities, whichever time is greater.

3.4.3 Notwithstanding any other provision of this Agreement, nothing in this Agreement shall be construed to modify the rights and benefits contained in the Covered Person's Coverage Agreement.

3.4.4 Provider and each Contracted Provider may not bill Covered Persons for Covered Services (except for deductibles, copayments, or coinsurance) where Payor denies payment because Provider or a Contracted Provider has failed to comply with the terms or conditions of this Agreement.

3.4.5 Provider and each Contracted Provider further agree (i) that the provisions of 3.4.1, 3.4.2, 3.4.3, 3.4.4 and 3.4.5 of this Section 3.4 shall survive termination of this Agreement regardless of the cause giving rise to termination and shall be construed to be for the benefit of Covered Persons, and (ii) that these provisions supersede any oral or written contrary agreement now existing or hereafter entered into between Contracted Provider and Covered Persons or persons acting on their behalf.

3.4.6 If Provider or Contracted Provider contracts with other providers or facilities who agree to provide Covered Services to Covered Persons with the expectation of receiving payment directly or indirectly from Payor, such providers or facilities must agree to abide by the provisions of Subsections 3.4.1 through 3.4.7.

3.4.7 Provider acknowledges that willfully collecting or attempting to collect payment from a Covered Person, knowing that collection to be in violation of this Section 3.4, constitutes a class C felony under RCW 48.80.030(5).

3.5. Terms and Conditions of Payment.

3.5.1 Payor shall pay Provider and each Contracted Provider for Covered Services in accordance with the applicable Compensation Schedule as soon as practical but subject to the following minimum standards: (a) Ninety-five percent (95%) of the monthly volume of Clean Claims shall be paid within thirty (30) days of receipt by Payor; (b) Ninety-five percent (95%) of the monthly volume of all claims shall be paid or denied within sixty (60) days of receipt by Payor, except as agreed to in writing by the Parties on a claim-by-claim basis. The date of receipt of a claim is the date the Payor or its agent receives either written or electronic notice of the claim. Payor shall utilize a reasonable method for confirming receipt of claims and responding to Provider or Contracted Provider inquiries thereof.

3.5.2 Failure to pay claims within these minimum standards will result in interest payments on undened and unpaid Clean Claims more than sixty-one (61) days old until Payor meets the standards in this Section 3.5. Interest shall be assessed at the rate of one percent (1%) per month, and shall be calculated monthly as simple interest prorated for any portion of a month. Payor shall add the interest payable to the amount of the unpaid claim without the necessity of the Provider or Contracted Provider submitting an additional claim. Any

interest paid under this Section shall not be applied by the Payor to a Covered Person's deductible, copayment, coinsurance, or any similar obligation of the Covered Person.

3.5.3 When Payor issues payment in Provider or Contracted Provider and Covered Person names, Payor shall make claim checks payable in the name of Provider or Contracted Provider first and Covered Person second.

3.5.4 Claim denials shall be communicated to Provider or Contracted Provider and shall include the specific reason why the claim was denied. If the denial is based upon Medical Necessity or similar grounds, then Payor upon request of Provider or Contracted Provider must also promptly disclose the supporting basis for the decision.

3.5.5 Payor shall be responsible for ensuring that any person acting on behalf of or at the direction of Payor or acting pursuant to Payor standards or requirements complies with these billing and claim payment standards.

3.5.6 The standards in this Section 3.5 do not apply in the following circumstances: to claims about which there is substantial evidence of fraud or misrepresentation by Provider, Contracted Providers or Covered Persons; in instances where Payor or Company has not been granted reasonable access to information under Contracted Provider's control; or if the failure to comply is occasioned by any act of God, bankruptcy, act of a governmental authority responding to an act of God or other emergency, or the result of a strike, lockout, or other labor dispute.

3.6. Recovery Rights - Payor. Payor or its delegate shall have the right to immediately offset or recoup any and all amounts owed by Provider or a Contracted Provider to Payor or Company against amounts owed by the Payor or Company to the Provider or Contracted Provider. Provider and Contracted Providers agree that all recoupment and any offset rights under this Agreement will constitute rights of recoupment authorized under State or federal law and that such rights will not be subject to any requirement of prior or other approval from any court or other government authority that may now have or hereafter have jurisdiction over Provider or a Contracted Provider. Notwithstanding the foregoing, except in the case of fraud, a Payor may not request (a) a refund of a payment previously made to satisfy a claim unless Payor does so in writing within twenty-four (24) months (or within thirty (30) months for reasons related to coordination of benefits) in accordance with RCW 48.43.600 or (b) payment of a contested refund sooner than six (6) months after receipt of the request. This section is not applicable to subrogation claims.

3.7. Recovery Rights - Provider. Except in the case of fraud, Provider or a Contracted Provider may not request payment from Company or Payor to satisfy a claim unless it does so in writing within twenty-four (24) months after the date the claim was denied or payment intended to satisfy the claim was made. In the case of coordination of benefits, Provider or a Contracted Provider must request from Company or Payor within thirty (30) months after original payment was made any additional balances owed. Additional payment cannot be requested any sooner than six (6) months after request is made. This section is not applicable to subrogation claims.

3.8. Out-of-Network Payments: In accordance with RCW 48.49.030 and RCW 48.49.040, Health Plan will negotiate in good faith with out-of-network providers/facilities to determine a commercially reasonable payment amount for services. Enrollees may not be held responsible for anything above their in-network cost share. Health Plan and out-of-network providers/facilities may pursue arbitration to determine a commercially reasonable payment amount as a dispute resolution process if good faith negotiations do not yield successful results.

3.9. Telemedicine Payment Parity: In accordance with RCW 48.43.735, Providers for telemedicine services shall be compensated at the same rate (to be defined by the Legislature) as in-person services. Health Plan can negotiate a telemedicine reimbursement rate that differs from in-person services for:

- hospitals,
- hospital systems,
- telemedicine companies, and

- provider groups consisting of 11 or more providers.

Health Plan can negotiate payment of facility fees for telemedicine services that originate at:

- a hospital,
- a rural health clinic,
- a Federally qualified health center,
- a physician/health care provider's office,
- community mental health center, skilled nursing facility, or
- a renal dialysis center (except an internal renal dialysis center).

Any other site may not charge a facility fee. Health Plan shall not distinguish between originating sites that are rural and urban when providing coverage. Health Plan is not required to reimburse:

- an originating site for professional fees,
- services not covered under the plan, or
- an originating site or provider that is not contracted under the plan.

Audio-only telemedicine billing requires advance consent from the patient. Failure to obtain patient consent could result in disciplinary action against the provider.

ARTICLE IV – RECORDS AND INSPECTIONS

4.1. Records. Each Contracted Provider shall maintain medical, financial and administrative records related to items or services provided to Covered Persons, including but not limited to a complete and accurate permanent medical record for each such Covered Person, in such form and detail as are required by applicable Regulatory Requirements and consistent with generally accepted medical standards. Such records shall be maintained for a minimum of ten (10) years after final payment is made under this Agreement. However, when an audit, litigation, or other action involving records is initiated prior to the end of said period, records shall be maintained for a minimum of ten (10) years following resolution of such action. Medical records must support claims submitted to Company for payment in accordance with accepted standards for claims coding as interpreted and applied by the Payor and regulatory authorities.

4.2. Access. Provider and each Contracted Provider shall provide access to their respective books and records to each of the following, including any delegate or duly authorized agent thereof, subject to applicable Regulatory Requirements: (i) Company and Payor, during regular business hours and upon prior notice; (ii) appropriate State and federal authorities, to the extent such access is necessary to comply with Regulatory Requirements; and (iii) accreditation organizations. Access to health information and other similar records by Company or Payor shall be limited to records related to Covered Persons. Access to medical records for audit purposes must be limited to only that necessary to perform the audit. Provider and each Contracted Provider shall provide copies of such records at no expense to any of the foregoing that may make such request. Each Contracted Provider also shall obtain any authorization or consent that may be required from a Covered Person in order to release medical records and information to Company or Payor or any of their delegates. Provider and each Contracted Provider shall cooperate in and allow on-site inspections of its, his or her facilities and records by any Company, Payor, their delegates, any authorized government officials, and accreditation organizations. Provider and each Contracted Provider shall compile information necessary for the expeditious completion of such on-site inspection in a timely manner. Contracted Providers may audit or examine Company or Payor's books and records of account related to transactions between Company or Payor and Contracted Provider during normal business hours and upon reasonable prior notice.

4.3. Record Transfer. Subject to applicable Regulatory Requirements, each Contracted Provider shall cooperate in the timely transfer of Covered Persons' medical records to any other health care provider, at no charge and when required by Company.

ARTICLE V – INSURANCE AND INDEMNIFICATION

5.1. Insurance. During the term of this Agreement and for any applicable continuation period as set forth in Section 7.3 of this Agreement, Provider and each Contracted Provider shall maintain policies of general and professional liability insurance and other insurance necessary to insure Provider and such Contracted Provider, respectively; their respective employees; and any other person providing services hereunder on behalf of Provider or such Contracted Provider, as applicable, against any claim(s) of personal injuries or death alleged to have been caused or caused by their performance under this Agreement. Such insurance shall include, but not be limited to, any “tail” or prior acts coverage necessary to avoid any gap in coverage. Insurance shall be through a licensed carrier acceptable to Health Plan, and in a minimum amount of one million dollars (\$1,000,000) per occurrence, and three million dollars (\$3,000,000) in the aggregate unless a lesser amount is accepted by Health Plan or where State law mandates otherwise. Provider and each Contracted Provider will provide Health Plan with at least fifteen (15) days’ prior written notice of cancellation, non-renewal, lapse, or adverse material modification of such coverage. Upon Health Plan’s request, Provider and each Contracted Provider will furnish Health Plan with evidence of such insurance.

5.2. Indemnification by Provider and Contracted Provider. Provider and each Contracted Provider shall indemnify and hold harmless (and at Health Plan’s request defend) Company and Payor and all of their respective officers, directors, agents and employees from and against any and all third party claims for any loss, damages, liability, costs, or expenses (including reasonable attorney’s fees) judgments or obligations arising from or relating to any negligence, wrongful act or omission, or breach of this Agreement by Provider, a Contracted Provider, or any of their respective officers, directors, agents or employees.

5.3. Indemnification by Health Plan. Health Plan agrees to indemnify and hold harmless (and at Provider’s request defend) Provider, Contracted Providers, and their officers, directors, agents and employees from and against any and all third party claims for any loss, damages, liability, costs, or expenses (including reasonable attorney’s fees), judgments, or obligations arising from or relating to any negligence, wrongful act or omission or breach of this Agreement by Company or its directors, officers, agents or employee.

ARTICLE VI – DISPUTE RESOLUTION

6.1. Informal Dispute Resolution. Any dispute between Provider and/or a Contracted Provider, as applicable (the “Provider Party”), and Health Plan and/or Company, as applicable (including any Company acting as Payor) (the “Administrator Party”), with respect to or involving the performance under, termination of, or interpretation of this Agreement, or any other claim or cause of action hereunder, whether sounding in tort, contract or under statute (a “Dispute”) shall first be addressed by exhausting the applicable policies and procedures pertaining to claims payment, credentialing, utilization management, or other programs. If, at the conclusion of these applicable procedures, the matter is not resolved to the satisfaction of the Provider Party and the Administrator Party, or if there are no such policies, then the Provider Party and the Administrator Party agree that they will engage in a period of good faith negotiations between their designated representatives who have authority to settle the Dispute, which negotiations may be initiated by either the Provider Party or the Administrator Party upon written request to the other, provided such request takes place within one year of the date on which the requesting party first had, or reasonably should have had, knowledge of the event(s) giving rise to the Dispute. Administrator Party must render a decision on a Provider Party complaint within a reasonable time for the type of dispute. In the case of billing disputes, the Administrator Party must render a decision within thirty (30) days of a complaint. If a written complaint is submitted by the Provider Party and the Administrator Party does not grant or reject such written complaint within 30 days, then the Provider Party may, but is not required to, choose to proceed as if the complaint is rejected and submit the complaint to nonbinding mediation. Alternatively, if the matter has not been resolved within sixty (60) days of such request, either the Provider Party or the Administrator Party may initiate arbitration pursuant to Section 6.2 below by providing a written request to the other party. The other party may, but is not required to, consent to such binding arbitration process.

6.2. Arbitration. If mutually agreed upon by the Provider Party and the Administrator Party, either of the Provider Party and the Administrator Party wishing to pursue the Dispute as provided in Section 6.1 may submit it to binding arbitration conducted in accordance with the Commercial Arbitration Rules of the American Arbitration Association (“AAA”). In no event may any arbitration be initiated more than one (1) year following, as applicable, the end of the thirty (30) day negotiation period set forth in Section 6.1, or the date of notice of termination. Arbitration proceedings shall be conducted by an arbitrator chosen from the National Healthcare Panel at a mutually agreed upon location within the State. Each of the Provider Party and the Administrator Party shall bear its own costs and attorneys’ fees related to the arbitration except that the AAA’s Administrative Fees, all Arbitrator Compensation and travel and other expenses, and all costs of any proof produced at the direct request of the arbitrator shall be borne equally by the applicable parties, and the arbitrator shall not have the authority to order otherwise. The existence of a Dispute or arbitration proceeding shall not in and of itself constitute cause for termination of this Agreement. Except as hereafter provided, during an arbitration proceeding, each of the Provider Party and the Administrator Party shall continue to perform its obligations under this Agreement pending the decision of the arbitrator. Nothing herein shall bar either the Provider Party or the Administrator Party from seeking emergency injunctive relief to preclude any actual or perceived breach of this Agreement. Nothing contained in this Article VI shall limit a Party’s right to terminate this Agreement with or without cause in accordance with Section 7.2. Nothing herein shall be construed to require alternative dispute resolution to the exclusion of judicial remedies.

ARTICLE VII – TERM AND TERMINATION

7.1. Term. This Agreement is effective as of the Effective Date, and will remain in effect for an initial term (“Initial Term”) of one (1) years, after which it will automatically renew for successive terms of one (1) year each (each a “Renewal Term”), unless this Agreement is sooner terminated as provided in this Agreement or either Party gives the other Party written notice of non-renewal of this Agreement not less than one hundred eighty (180) days prior to the end of the then-current term.

7.2. Termination. This Agreement, or the participation of Provider or a Contracted Provider as a Participating Provider in one or more Products, may be terminated or suspended as set forth below.

7.2.1. Upon Notice. This Agreement may be terminated by either Party giving the other Party at least one hundred eighty (180) days’ prior written notice of such termination.

7.2.2. With Cause. This Agreement, or the participation of any Contracted Provider as a Participating Provider in one or more Products under this Agreement, may be terminated by either Party giving at least ninety (90) days’ prior written notice of termination to the other Party if such other Party (or the applicable Contracted Provider) is in breach of any material term or condition of this Agreement and such other Party (or the Contracted Provider) fails to cure the breach within the sixty (60) day period immediately following the giving of written notice of such breach. Any notice given pursuant to this Section 7.2.2 must describe the specific breach. In the case of a termination of a Contracted Provider, Provider shall immediately notify the affected Contracted Provider of such termination.

7.2.3. Suspension of Participation. Unless expressly prohibited by applicable Regulatory Requirements, Health Plan has the right to immediately suspend or terminate the participation of a Contracted Provider in any or all Products by giving written notice thereof to Provider when Health Plan determines that (i) based upon available information, the continued participation of the Contracted Provider appears to constitute an immediate threat or risk to the health, safety or welfare of Covered Persons, or (ii) the Contracted Provider’s fraud, malfeasance or non-compliance with Regulatory Requirements is reasonably suspected. Provider shall immediately notify the affected Contracted Provider of such suspension. During such suspension, the Contracted Provider shall, as directed by Health Plan, discontinue the provision of all or a particular Covered Service to Covered Persons. During the term of any suspension, the Contracted Provider shall notify Covered Persons that his or her status as a Participating Provider has been suspended. Such suspension will continue until the Contracted Provider’s participation is reinstated or terminated.

7.2.4. Insolvency. This Agreement may be terminated immediately by a Party giving written notice thereof to the other Party if the other Party is insolvent or has bankruptcy proceedings initiated against it.

7.2.5. Credentialing. The status of a Contracted Provider as a Participating Provider in one or more Products may be terminated immediately by Health Plan giving written notice thereof to Provider if the Contracted Provider fails to adhere to Health Plan's credentialing criteria, including, but not limited to, if the Contracted Provider (i) loses, relinquishes, or has materially affected its license to provide Covered Services in the State, (ii) fails to comply with the insurance requirements set forth in this Agreement; or (iii) is convicted of a criminal offense related to involvement in any state or federal health care program or has been terminated, suspended, barred, voluntarily withdrawn as part of a settlement agreement, or otherwise excluded from any state or federal health care program. Provider shall immediately notify the affected Contracted Provider of such termination. _

7.3. Effect of Termination. After the effective date of termination of this Agreement or a Contracted Provider's participation in a Product, this Agreement shall remain in effect for purposes of those obligations and rights arising prior to the effective date of termination. Upon such a termination, each affected Contracted Provider (including Provider, if applicable) shall (i) continue to provide Covered Services to Covered Persons in the applicable Product(s) during the longer of the ninety (90) day period following the date of such termination or such other period as may be required under any Regulatory Requirements, and, if requested by Company, each affected Contracted Provider (including Provider, if applicable) shall continue to provide, as a Participating Provider, Covered Services to Covered Persons until such Covered Persons are assigned or transferred to another Participating Provider in the applicable Product(s), and (ii) continue to comply with and abide by all of the applicable terms and conditions of this Agreement, including, but not limited to, Section 3.4 (Hold Harmless) hereof, in connection with the provision of such Covered Services during such continuation period. During such continuation period, each affected Contracted Provider (including Provider, if applicable) will be compensated in accordance with this Agreement and shall accept such compensation as payment in full. Company shall make a good faith effort to provide written notice of termination within fifteen (15) working days of receipt or issuance of a notice of termination to all Covered Persons who are patients seen on a regular basis by each affected Contracted Provider that is terminated, regardless of the cause for termination. The Contracted Provider will inform any Covered Person that seeks the Contracted Provider's services that this Agreement has been terminated.

7.4. Survival of Obligations. All provisions hereof that by their nature are to be performed or complied with following the expiration or termination of this Agreement, including without limitation Sections 2.8, 2.10, 3.2, 3.4, 3.5, 3.6, 3.7, 4.2, 4.3, 5.1, 5.2, 5.3, 7.3, and 7.4 and Article VIII, survive the expiration or termination of this Agreement.

ARTICLE VIII - MISCELLANEOUS

8.1. Relationship of Parties. The relationship between or among Health Plan, Company, Provider, and any Contracted Provider hereunder is that of independent contractors. None of the provisions of this Agreement will be construed as creating any agency, partnership, joint venture, employee-employer, or other relationship.

8.2. Conflicts Between Certain Documents. If there is any conflict between this Agreement and any policy or procedure of Company, this Agreement will control. In the event of any conflict between this Agreement and any Product Attachment, the Product Attachment will control as to such Product.

8.3. Assignment. This Agreement is intended to secure the services of and be personal to Provider and may not be assigned, sublet, delegated or transferred by Provider without Health Plan's prior written consent. Health Plan shall have the right, exercisable in its sole discretion, to assign or transfer all or any portion of its rights or to delegate all or any portion of its interests under this Agreement or any Attachment to an Affiliate, successor of Health Plan, or purchaser of the assets or stock of Health Plan, or the line of business or business unit primarily responsible for carrying out Health Plan's obligations under this Agreement.

8.4. Headings. The headings of the sections of this Agreement are inserted merely for the purpose of convenience and do not limit, define, or extend the specific terms of the section so designated.

8.5. Governing Law. The interpretation of this Agreement and the rights and obligations of Health Plan, Company, Provider and any Contracted Providers hereunder will be governed by and construed in accordance with applicable federal and State laws.

8.6. Third Party Beneficiary. This Agreement is entered into by the Parties signing it for their benefit, as well as, in the case of Health Plan, the benefit of Company, and in the case of Provider, the benefit of each Contracted Provider. Except as specifically provided in Section 3.4 hereof, no Covered Person or third party, other than Company, will be considered a third party beneficiary of this Agreement.

8.7. Amendment. Except as otherwise provided in this Agreement, this Agreement may be amended only by written agreement of duly authorized representatives of the Parties.

8.7.1. Health Plan may amend this Agreement by giving Provider written notice of the amendment to the extent such amendment is deemed necessary or appropriate by Health Plan to comply with any Regulatory Requirements. Any such amendment will be deemed accepted by Provider upon the giving of such notice.

8.7.2. Health Plan may amend this Agreement by giving Provider written notice (electronic or paper) of the proposed amendment. Unless Provider notifies Health Plan in writing of its objection to such amendment during the thirty (30) day period following the giving of such notice by Health Plan, Provider shall be deemed to have accepted the amendment. If Provider objects to any proposed amendment to either the base agreement or any Attachment, Health Plan may exclude one or more of the Contracted Providers from being Participating Providers in the applicable Product (or any component program of, or Coverage Agreement in connection with, such Product).

8.7.3. Notwithstanding the above, Health Plan will give Provider at least sixty (60) days' prior written notice of any amendment or new Attachment involving changes that affect health care service delivery or compensation, unless changes to federal or State law or regulations make such advance notice impossible, in which case notice shall be provided as soon as possible. In such case, if Provider notifies Health Plan in writing of its objection to such amendment within thirty (30) days following the giving of such notice by Health Plan, such amendment or new Attachment shall not go into effect as to Provider; Health Plan may on sixty (60) days' notice terminate this Agreement or the participation of Provider and Contracted Providers in the Products affected by the proposed amendment (or any component program of such Products). No change to this Agreement will be made retroactive without the express consent of Provider.

8.8. Entire Agreement. All prior or concurrent agreements, promises, negotiations or representations either oral or written, between Health Plan and Provider relating to a subject matter of this Agreement, which are not expressly set forth in this Agreement, are of no force or effect.

8.9. Severability. The invalidity or unenforceability of any terms or provisions hereof will in no way affect the validity or enforceability of any other terms or provisions.

8.10. Waiver. The waiver by either Party of the violation of any provision or obligation of this Agreement will not constitute the waiver of any subsequent violation of the same or other provision or obligation.

8.11. Notices. Except as otherwise provided in this Agreement, any notice required or permitted to be given hereunder is deemed to have been given when such written notice has been personally delivered or deposited in the United States mail, postage paid, or delivered by a service that provides written receipt of delivery, addressed as follows:

To Health Plan at:

Attn: President

Coordinated Care Corporation

1145 Broadway, Suite 300

Tacoma, WA 98402

To Provider at:

Attn: Administrator

RadiantCare Physicians PLLC

4525 3rd Ave S, STE 100

Lacey, WA 98503

or to such other address as such Party may designate in writing. Notwithstanding the previous sentence, Health Plan may provide notices by electronic mail, through its provider newsletter or on its provider website.

8.12. Force Majeure. Neither Party shall be liable or deemed to be in default for any delay or failure to perform any act under this Agreement resulting, directly or indirectly, from acts of God, civil or military authority, acts of public enemy, war, accidents, fires, explosions, earthquake, flood, strikes or other work stoppages by either Party's employees, or any other similar cause beyond the reasonable control of such Party.

8.13. Proprietary Information. Each Party is prohibited from, and shall prohibit its Affiliates and Contracted Providers from, disclosing to a third party the substance of this Agreement, or any information of a confidential nature acquired from the other Party (or Affiliate or Contracted Provider thereof) during the course of this Agreement, except to agents of such Party as necessary for such Party's performance under this Agreement, or as required by a Payor Contract or applicable Regulatory Requirements. Provider acknowledges and agrees that all information relating to Company's programs, policies, protocols and procedures is proprietary information and Provider shall not disclose such information to any person or entity without Health Plan's express written consent.

8.14. Authority. The individuals whose signatures are set forth below represent and warrant that they are duly empowered to execute this Agreement. Provider represents and warrants that it has all legal authority to contract on behalf of and to bind all Contracted Providers to the terms of the Agreement with Health Plan.

* * * * *

[Signature Page Below]

IN WITNESS WHEREOF, the Parties hereto have executed this Agreement, including all Product Attachments noted on Schedule B, effective as of the date set forth beneath their respective signatures.

HEALTH PLAN:

Coordinated Care Corporation

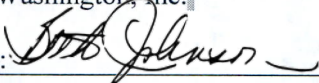
Authorized Signature: 

Print Name: Beth Johnson

Title: CEO

Signature Date: 01/17/2023

Coordinated Care of Washington, Inc.

Authorized Signature: 

Print Name: Beth Johnson

Title: CEO

Signature Date: 01/17/2023

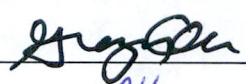
To be completed by Health Plan only:

Effective Date: 02/02/2023

PROVIDER:

RadiantCare Physicians PLLC

(Legibly Print Name of Provider)

Authorized Signature: 

Print Name: Gregory Allen

Title: Partner

Signature Date: 11/2/22

Tax Identification Number: 912018691

State Medicaid Number: 1020586

PARTICIPATING PROVIDER AGREEMENT

SCHEDULE A CONTRACTED PROVIDER-SPECIFIC PROVISIONS

Provider and Contracted Providers shall comply with the applicable provisions of this Schedule A. Applicable sections are indicated by a checked box where appropriate.

☐ 1 Hospitals. If Provider or a Contracted Provider is a hospital (“Hospital”), the following provisions apply.

1.1 24 Hour Coverage. Each Hospital shall be available to provide Covered Services to Covered Persons twenty-four (24) hours per day, seven (7) days per week.

1.2 Emergency Care. Each Hospital shall provide Emergency Care (as hereafter defined) in accordance with Regulatory Requirements. The Contracted Provider shall notify Company’s medical management department of any emergency room admissions by electronic file sent within twenty-four (24) hours or by the next business day of such admission. “Emergency Care” (or derivative thereof) has, as to each particular Product, the meaning set forth in the applicable Coverage Agreement or Product Attachment. If there is no definition in such documents, “Emergency Care” means inpatient and/or outpatient Covered Services furnished by a qualified provider that are needed to evaluate or stabilize an Emergency Medical Condition. “Emergency Medical Condition” means a medical condition manifesting itself by acute symptoms of sufficient severity (including severe pain) that a prudent layperson, who possesses an average knowledge of health and medicine, could reasonably expect the absence of immediate medical attention to result in the following: (i) placing the health of the individual (or, with respect to a pregnant woman, the health of the woman or her unborn child) in serious jeopardy; (ii) serious impairment to bodily functions; or (iii) serious dysfunction of any bodily organ or part.

1.3 Staff Privileges. Each Hospital shall assist in granting staff privileges or other appropriate access to Company’s Participating Providers who are qualified medical or osteopathic physicians, provided they meet the reasonable standards of practice and credentialing standards established by the Hospital’s medical staff and bylaws, rules, and regulations.

1.4 Discharge Planning. Each Hospital agrees to cooperate with Company’s system for the coordinated discharge planning of Covered Persons, including the planning of any necessary continuing care.

1.5 Credentialing Criteria. Each Hospital shall (a) currently, and for the duration of this Agreement, remain accredited by the Joint Commission or American Osteopathic Association, as applicable; and (b) ensure that all employees of Hospital perform their duties in accordance with all applicable local, State and federal licensing requirements and standards of professional ethics and practice.

1.6 National Committee for Quality Assurance (“NCQA”) Accreditation of Health Plans Standards. Each Hospital agrees to: i) cooperate with Quality Management and Improvement (“QI”) activities; ii) maintain the confidentiality of a Covered Persons information and records pursuant to the agreement; and iii) allow the Company to use Hospital’s performance data.

☒ 2 Practitioners. If Provider or Contracted Provider is a physician or other health care practitioner (including physician extenders) (“Practitioner”), the following provisions apply.

2.1 Contracted Professional Qualifications. At all times during the term of this Agreement, Practitioner shall, as applicable, maintain medical staff membership and admitting privileges with at least one hospital that is a Participating Provider (“Participating Hospital”) with respect to each Product in which the Practitioner participates. Upon Company’s request, Practitioner shall furnish evidence of the foregoing to Company. If Practitioner does not have such admitting privileges, Provider or the Practitioner shall provide

Company with a written statement from another Participating Provider who has such admitting privileges, in good standing, certifying that such individual agrees to assume responsibility for providing inpatient Covered Services to Covered Persons who are patients of the applicable Practitioner.

2.2 Acceptance of New Patients. To the extent that Practitioner is accepting new patients, such Practitioner must also accept new patients who are Covered Persons with respect to the Products in which such Practitioner participates. Practitioner shall notify Company in writing forty-five (45) days prior to such Practitioner's decision to no longer accept Covered Persons with respect to a particular Product. In no event will an established patient of any Practitioner be considered a new patient.

2.3 Preferred Drug List/Drug Formulary. If applicable to the Covered Person's coverage, Practitioners shall use commercially reasonable efforts, when medically appropriate under the circumstances, to comply with formulary or preferred drug list when prescribing medications for Covered Persons.

2.4 National Committee for Quality Assurance ("NCQA") Accreditation of Health Plans Standards. Each Practitioner agrees to: i) cooperate with Quality Management and Improvement ("QI") activities; ii) maintain the confidentiality of a Covered Persons information and records pursuant to the Agreement; and iii) allow the Company to use Practitioner's performance data.

☐ 3 Ancillary Providers. If Provider or Contracted Provider is an ancillary provider (including but not limited to a chemical dependency services provider, residential treatment facility/BH agency, home health agency, durable medical equipment provider, sleep center, pharmacy, ambulatory surgery center, nursing facility, laboratory or urgent care center)("Ancillary Provider"), the following provisions apply.

3.1 Acceptance of New Patients. To the extent that Ancillary Provider is accepting new patients, such Ancillary Provider must also accept new patients who are Covered Persons with respect to the Products in which such Ancillary Provider participates. Ancillary Provider shall notify Company in writing forty-five (45) days prior to such Ancillary Provider's decision to no longer accept Covered Persons with respect to a particular Product. In no event will an established patient of any Ancillary Provider be considered a new patient.

3.2 National Committee for Quality Assurance ("NCQA") Accreditation of Health Plans Standards. Each ancillary provider agrees to: i) cooperate with Quality Management and Improvement ("QI") activities; ii) maintain the confidentiality of a Covered Persons information and records pursuant to the Agreement; and iii) allow the Company to use ancillary provider's performance data

☐ 4 FQHC. If Provider or a Contracted Provider is a federally qualified health center ("FQHC"), the following provision applies.

4.1 FQHC Insurance. To the extent FQHC's employees are deemed to be federal employees qualified for protection under the Federal Tort Claims Act ("FTCA") and Health Plan has been provided with documentation of such status issued by the U.S. Department of Health and Human Services (such status to be referred to as "FTCA Coverage"), Section 5.1 of this Agreement will not apply to those Contracted Providers with FTCA Coverage. FQHC shall provide evidence of such FTCA Coverage to Health Plan at any time upon request. FQHC shall promptly notify Health Plan if, any time during the term of this Agreement, any Contracted Provider is no longer eligible for, or if FQHC becomes aware of any fact or circumstance that would jeopardize, FTCA Coverage. Section 5.1 of this Agreement will apply to a Contracted Provider immediately upon such Contracted Provider's loss of FTCA Coverage for any reason.

5. Facility Providers. If Provider or a Contracted Provider is a facility (including but not limited to Clinic, FQHC, LTAC, Nursing Home, Rehab, Rural Health Clinic, Skilled Nursing) ("Facility Provider") the following provision applies.

5.1 National Committee for Quality Assurance ("NCQA") Accreditation of Health Plans Standards. Each facility agrees to: i) cooperate with Quality Management and Improvement ("QI") activities; ii)

maintain the confidentiality of a Covered Persons information and records pursuant to the Agreement; and iii) allow the Company to use facility's performance data.



6. Long Term Services and Supports ("LTSS") Provider. If Provider or a Contracted Provider is a provider of LTSS, the following provisions apply.

6.1 Definition. LTSS generally includes assistance with daily self-care activities (e.g., walking, toileting, bathing, and dressing) and activities that support an independent lifestyle (e.g., food preparation, transportation, and managing medications). The broad category of LTSS also includes care and service coordination for people who live in their own home, a residential setting, a nursing facility, or other institutional setting. Home and community-based services ("HCBS") are a subset of LTSS that functions outside of institutional care to maximize independence in the community.

6.2 Acknowledgement. Health Plan acknowledges that Provider is a provider of LTSS and is not necessarily a provider of medical or health care services. Nothing in this Agreement is intended to require Provider to provide medical or health care services that Provider does not routinely provide, but would not prohibit providers from offering these services, as appropriate.

6.3 Notification Requirements. Provider or the applicable Contracted Provider shall provide the following notifications to Health Plan, via written notice or via telephone contact at a number to be provided by Health Plan, within the following time frames:

a) Provider or the applicable Contracted Provider shall notify Health Plan of a Covered Person's visit to urgent care or the emergency department of any hospital, or of a Covered Person's hospitalization, within 24 hours of becoming aware of such visit or hospitalization.

b) Provider or the applicable Contracted Provider shall notify Health Plan of any change to the designated/assigned services being provided under a Covered Person's plan of care and/or service plan, within 24 hours of becoming aware of such change.

c) Provider or the applicable Contracted Provider shall notify Health Plan if a Covered Person misses an appointment with Provider, within 24 hours of becoming aware of such missed appointment.

d) Provider or the applicable Contracted Provider shall notify Health Plan of any change in a Covered Person's medical or behavioral health condition, within 24 hours of becoming aware of such change. (Examples of changes in condition are set forth in the Health Plan Policies and Procedures.)

e) Provider or the applicable Contracted Provider shall notify Health Plan of any safety issue identified by Provider or Contracted Provider or its agent or subcontractor, within 24 hours of the identification of such safety issue. (Examples of safety issues are set forth in the Health Plan Policies and Procedures.)

f) Provider or the applicable Contracted Provider shall notify Health Plan of any change in Provider's or Contracted Provider's key personnel, within 24 hours of such change.

6.4 Minimum Data Set. If Contracted Provider is a nursing facility, Provider or such Contracted Provider shall submit to Health Plan or its designee the Minimum Data Set as defined by CMS and required under federal law and Health Plan policy as it relates to all Covered Persons who are residents in Contracted Provider's facility. Such submission shall be via electronic mail, facsimile transmission, or other manner and format reasonably requested by Health Plan.

6.5 Quality Improvement Plan. Each Contracted Provider shall participate in Health Plan's LTSS quality improvement plan. Each Contracted Provider shall permit Health Plan to access such Contracted Providers' assessment and quality data upon reasonable advance notice, which may be given by electronic mail.

6.6 Electronic Visit Verification. If Contracted Provider provides in-home services, Contracted Provider shall comply with Health Plan's electronic visit verification system requirements where applicable and accessible.

6.7 Criminal Background Checks. Provider shall conduct a criminal background check on each Contracted Provider prior to the commencement of services under this Agreement and as requested by Health Plan thereafter. Provider shall provide the results of such background checks to Health Plan and member, if self-directed, upon request. Provider agrees to immediately notify Health Plan of any criminal convictions of any Contracted or sub-contracted Provider. Provider shall pay any costs associated with such criminal background checks.

PARTICIPATING PROVIDER AGREEMENT

SCHEDULE B PRODUCT PARTICIPATION

Provider and Contracted Providers will be subject to and bound by the Attachments marked below, and designated as a “Participating Provider” in the Products marked below as of the date of successful completion of credentialing in accordance with this Agreement.

List of Product Attachments:

X	Attachment A: Medicaid		
	X	Exhibit A-1	Medicaid Product Attachment
		Exhibit A-2	Medicaid Hosp. Comp. Sched.
	X	Exhibit A-3	Medicaid Pract. Comp. Sched.
		Exhibit A-3A	Medicaid Pract. Comp. Sched. RFP
		Exhibit A-4	Medicaid Vision Comp. Sched.
		Exhibit A-5	Medicaid SNF Comp. Sched.
		Exhibit A-6	Medicaid Ancillary Comp. Sched.
		Exhibit A-7	Medicaid Critical Access Hosp. Comp. Sched.
		Exhibit A-8	Medicaid Ancillary – DME Comp. Sched.
		Exhibit A-9	Medicaid LTC and Rehab. Comp. Sched.
		Exhibit A-10	Medicaid Ancillary – Dialysis Comp. Sched.
		Exhibit A-11	Medicaid Ancillary – ASC Comp. Sched.
		Exhibit A-12	Medicaid Ancillary BH Comp. Sched.
		Exhibit A-12A	Medicaid Ancillary BH Comp. Sched. RFP
		Value-Based Purchasing	
		Exhibit A-13.1	Medicaid VBP HBR
		Exhibit A-13.2	Medicaid VBP QBF
		Exhibit A-13.3	Medicaid VBP Care Management Fee
		Exhibit A-14	Medicaid Wraparound with Intensive Services Comp.
		[Reserved for future Exhibits]	

	Attachment B: Medicare		
		Exhibit B-1	Medicare Product Attachment
		MA-MA-PD Plan	
		Exhibit B-2.1	MA/MA-PD Hospital Comp. Sched.
		Exhibit B-2.2	MA/MA-PD Practitioner Comp. Sched.
		Exhibit B-2.3	MA/MA-PD Vision Comp. Sched.
		Exhibit B-2.4	MA/MA-PD Critical Access Hosp. Comp. Sched.
		Exhibit B-2.5	MA/MA-PD Facility - LTC, Rehab., SNF Comp. Sched.
		Exhibit B-2.6	MA/MA-PD Ancillary - Amb., HH, Hospice, Lab. Comp. Sched.

_____	Exhibit B-2.7	MA/MA-PD Ancillary - ASC Comp. Sched.
_____	Exhibit B-2.8	MA/MA-PD Ancillary - Dialysis Comp. Sched.
_____	Exhibit B-2.9	MA/MA-PD Ancillary - DME Comp. Sched.
_____	[Reserved for future Exhibits]	

_____	DSNP Plan	
_____	Exhibit B-3.1	DSNP Hospital Comp. Sched.
_____	Exhibit B-3.2	DSNP Practitioner Comp. Sched.
_____	Exhibit B-3.3	DSNP Vision Comp. Sched.
_____	Exhibit B-3.4	DSNP Critical Access Hosp. Comp. Sched.
_____	Exhibit B-3.5	DSNP Facility - LTC, Rehab., SNF Comp. Sched.
_____	Exhibit B-3.6	DSNP Ancillary - Amb., HH, Hospice, Lab. Comp. Sched.
_____	Exhibit B-3.7	DSNP Ancillary - ASC Comp. Sched.
_____	Exhibit B-3.8	DSNP Ancillary - Dialysis Comp. Sched.
_____	Exhibit B-3.9	DSNP Ancillary - DME Comp. Sched.
_____	[Reserved for future Exhibits]	

X	Attachment C: Commercial-Exchange		
	X	Exhibit C-0	CE Product Attachment
	_____	Exhibit C-1	CE Hosp. Comp. Sched.
	X	Exhibit C-2	CE Pract. Comp. Sched.
	_____	Exhibit C-3	CE Vision Comp. Sched.
	_____	Exhibit C-4	CE SNF Comp. Sched.
	_____	Exhibit C-5	CE Ancillary Comp. Sched.
	_____	Exhibit C-6A	CE Critical Access Hosp. Comp. Sched. A
	_____	Exhibit C-6B	CE Critical Access Hosp. Comp. Sched. B
	_____	Exhibit C-7	CE Ancillary – DME Comp. Sched.
	_____	Exhibit C-8	CE LTC and Rehab. Comp. Sched.
	_____	Exhibit C-9	CE Ancillary – Dialysis Comp. Sched.
	_____	Exhibit C-10	CE Ancillary – ASC Comp. Sched.
	_____	Exhibit C-11	CE Ancillary BH Comp. Sched.
	_____	[Reserved for future Exhibits]	

_____	Attachment D1: Indian Healthcare Provider Addendum
_____	Attachment D2: Medicaid and CHIP Managed Care Addendum for IHCPs
_____	Attachment E: Delegated Credentialing Agreement

		Exhibit E-1	Delegated Credentialing Agreement (CCC)
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	Attachment F: Medicaid – Foster Care Program
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	Exhibit F-1	AHFC Medicaid Product Attachment
	Exhibit F-2	AHFC Medicaid Hospital Comp. Sched.
	Exhibit F-3	AHFC Medicaid Practitioner Comp. Sched.
	Exhibit F-4	AHFC Medicaid Vision Comp. Sched.
	Exhibit F-5	AHFC Medicaid SNF Comp. Sched.
	Exhibit F-6	AHFC Medicaid Ancillary Comp. Sched.
	Exhibit F-7	AHFC Medicaid Critical Access Hosp. Comp. Sched.
	Exhibit F-8	AHFC Medicaid Ancillary - DME Comp. Sched.
	Exhibit F-9	AHFC Medicaid LTC and Rehab. Comp. Sched.
	Exhibit F-10	AHFC Medicaid Ancillary - Dialysis Comp. Sched.
	Exhibit F-11	AHFC Medicaid Ancillary - ASC Comp. Sched.
	Exhibit F-12	AHFC Medicaid Ancillary BH Comp. Sched.

[Reserved for future Exhibits]

X	Attachment G: Cascade Care Public Option Program
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X	Exhibit G-0	Cascade Care Product Attachment
	Exhibit G-1	Cascade Care Hosp. Comp. Sched.
X	Exhibit G-2	Cascade Care Pract.Comp. Sched.
	Exhibit G-3	Cascade Care Vision Comp. Sched.
	Exhibit G-4	Cascade Care SNF Comp. Sched.
	Exhibit G-5	Cascade Care Ancillary Comp. Sched.
	Exhibit G-6a	Cascade Care Critical Access Hospital Comp. Sched. A
	Exhibit G-6b	Cascade Care Critical Access Hospital Comp. Sched. B
	Exhibit G-7	Cascade Care Ancillary – DME Comp. Sched.
	Exhibit G-8	Cascade Care LTC and Rehab. Comp. Sched.
	Exhibit G-9	Cascade Care Ancillary – Dialysis Comp. Sched.
	Exhibit G-10	Cascade Care Ancillary – ASC Comp. Sched.
	Exhibit G-11	Cascade Care Ancillary BH Comp. Sched.

[Reserved for future Exhibits]

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[Reserved for future Exhibits]

Health Plan to which Product Attachments Apply:

Each Attachment and related Exhibits checked above shall be a Product of Coordinated Care Corporation (“CCC”) from the Effective Date of this Agreement. On at least sixty (60) days’ advance written notice (written or electronic), CCC and Coordinated Care of Washington, Inc. (“CCW”) may jointly notify Provider that an Attachment (and related Exhibits if any) shall no longer be a Product of CCC and shall be a Product of CCW on the date specified in the notice. As of such date, the designated Attachment(s) (and related Exhibits if any) shall terminate with respect to CCC as the Health Plan and commence with respect to CCW as the Health Plan for such Product.

PARTICIPATING PROVIDER AGREEMENT

SCHEDULE C

CONTRACTED PROVIDERS

[illegible]

NOTE: This Schedule is intended to capture all groups, clinics and facilities associated with the Agreement at the time of contract signature; entities may be added or removed from time to time, in accordance with the Agreement. If the Agreement is for an individual practitioner, no roster is required; however, please enter the individual practitioner's information in the table, above.

ATTACHMENT A: Medicaid

EXHIBIT A-1 APPLE HEALTH PARTICIPATING PROVIDER AGREEMENT ATTACHMENT

This Apple Health Participating Provider Agreement Attachment (this “**Attachment**”) is incorporated into the Participating Provider Agreement (the “**Agreement**”) entered into by and between RadiantCare Physicians PLLC (“**Provider**”), an entity described more fully in the signature block, and Coordinated Care of Washington, Inc., a health care service contractor (“Health Plan”).

Note that this Attachment is based on the 2020 IMC Apple Health contract.

Network: All Contracted Providers under this Agreement will participate as Participating Providers in and become part of the CCCWA network.

ARTICLE I RECITALS

1.1 Health Plan has contracted with HCA to arrange for the provision of integrated physical and behavioral health care services to Covered Persons under the Apple Health Program.

1.2 This Attachment is intended to supplement the Agreement by setting forth the parties’ rights and responsibilities related to the provision of Covered Services to Covered Persons as it pertains to the Apple Health Program. In the event of a conflict between the terms and conditions of the Agreement and the terms and conditions of this Attachment, this Attachment shall govern as to the Apple Health Program.

1.3 Provider agrees and understands that Covered Services shall be provided in accordance with the State Contract(s), Payor requirements, any applicable State handbooks or policy and procedure guides, and all applicable State and federal laws and regulations. To the extent Provider is unclear about Provider’s duties and obligations, Provider shall request clarification from Health Plan.

ARTICLE II DEFINITIONS

Capitalized terms used and not otherwise defined herein shall have the meanings given to them in the Agreement or the State Contract. The definitions listed below will supersede any meanings contained elsewhere in the Agreement with regard to this Attachment.

2.1 **Apple Health Program** shall mean the Medicaid managed care program known as Apple Health, including both non-integrated managed care and integrated managed care, as applicable based on service area.

2.2 **Covered Person** shall have the meaning set forth in the Agreement.

2.3 **HCA** means the State of Washington Health Care Authority and its employees and authorized agents.

2.4 **Medically Necessary** means health care services that: (a) are reasonably calculated to prevent, diagnose, correct, cure, alleviate or prevent worsening of conditions in the Covered Person that endanger life, or cause suffering or pain, or result in an illness or infirmity, or threaten to cause or aggravate a handicap, or cause physical deformity or malfunction; and (b) are not more costly than any other equally effective or more conservative course of treatment available or suitable for the Covered Person requesting the service. Such services shall include services related to the Covered Person’s ability to achieve age-appropriate growth and development.

2.5 ***Physician's Orders for Life Sustaining Treatment ("POLST")*** means a set of guidelines and protocols for how emergency medical personnel shall respond when summoned to the site of an injury or illness for the treatment of a person who has signed a written directive or durable power of attorney requesting that he or she not receive futile emergency medical treatment, in accordance with RCW 43.70.480.

2.6 ***Primary Care Provider or PCP*** means a Participating Provider who has the responsibility for supervising, coordinating, and providing primary health care to Covered Persons, initiating referrals for specialist care, and maintaining the continuity of Covered Person care. PCPs include, but are not limited to pediatricians, family practitioners, general practitioners, internists, naturopathic physicians, medical residents (under the supervision of a teaching physician), physician assistants (under the supervision of a physician), or advanced registered nurse practitioners (nurse practitioners), as designated by Health Plan. The definition of PCP is inclusive of primary care physician as it is used in 42 C.F.R. § 438. All Federal requirements applicable to primary care physicians will also be applicable to PCPs as the term is used in this Attachment.

2.7 ***State*** means the state of Washington.

2.8 ***State Contract(s)*** means the applicable contract(s) between HCA and Health Plan under which Health Plan agrees to provide or arrange for services related to the Apple Health Program, including any exhibits, attachments, documents, or materials incorporated by reference.

ARTICLE III PROVIDER CONTRACT REQUIREMENTS

3.1 **Health Plan Remains Legally Responsible.** Nothing herein shall be construed to delegate legal responsibility to HCA for any work performed under the Agreement, nor for oversight of any functions and/or responsibilities delegated to Provider. [Source: § 9.1 of the 2020 Apple Health IMC Agreement]

3.2 **Compliance with Applicable Law.** Provider shall comply with all Applicable Law. For purposes of this Attachment, Applicable Law shall specifically include those laws and regulations as set forth in the State Contract, including but not limited to the Health Insurance Portability and Accountability Act of 1996 ("HIPAA"), the Mental Health Parity and Addiction Equity Act ("MHPAEA") and final rule, The 21st Century Cures Act, state laws and regulations regarding mental and behavioral health and substance use disorder services, the Federal Drug and Alcohol Confidentiality Laws in 42 C.F.R. Part 2, 42 U.S.C. §§ 1396a(a)(43) (early and periodic screening, diagnostic, and treatment services ("EPSDT")), 1396d(r) (definition of EPSDT), and 42 C.F.R. § 438.3(l) (choice of network provider). [Source: §§ 2.4, 9.4.12 of the 2020 Apple Health IMC Agreement]

3.3 **Compliance with State Contract.** Provider shall comply with any term or condition of the State Contract that is applicable to the services to be performed under the Agreement, including but not limited to the Performance Improvement Project requirements of the State Contract and the prohibition on direct and/or indirect door-to-door, telephonic, or other cold-call marketing. [Source: §§ 7.2, 3.1.8 of the 2020 Apple Health IMC Agreement]

3.4 **Policies and Procedures.** Provider shall comply with Health Plan's policies and procedures, including, but not limited to, credentialing and recredentialing; utilization management; fraud, waste and abuse; authorization of services; quality improvement activities; and provider payment suspensions. Provider shall comply with the Program Integrity requirements of the State Contract, as well as Health Plan's program integrity policies and procedures. Without limiting the generality of the foregoing, Provider shall comply with the Program Integrity requirements in Section 1902(a)(68) of the Social Security Act, 42 C.F.R. § 438.610, 42 C.F.R. § 455, 42 C.F.R. § 1000 through 1008, and Chapter 182-502A WAC. Further, Provider shall be subject to ongoing analysis of utilization, claims, billing and/or encounter data to detect overpayment, which analysis shall include audits and investigations of Provider. To the extent that Provider is delegated authority for authorization of services, Provider shall comply with all Utilization Management requirements described in the State Contract. [Source: § 7, 11, 12, 12.5, 12.6 of the 2020 Apple Health IMC Agreement]

3.5 Debarment Certification. Provider represents and warrants that it is not presently debarred, suspended, proposed for debarment, declared ineligible, or voluntarily excluded by any State or federal department or agency from participating in transactions. Provider shall immediately notify Health Plan in writing if, during the term of the Agreement, (a) Provider becomes debarred, suspended, proposed for debarment, declared ineligible or voluntarily excluded, or (b) Provider or any of Provider's employees are subject to disciplinary action against accreditation, certification, license and/or registration. Further, Provider shall not pay for goods and services furnished by an excluded person, at the medical direction, or on the prescription of an excluded person. [Source: § 12.8, 12.8.1 of the 2020 Apple Health IMC Agreement]

3.6 Records. Provider shall maintain all financial, billing, medical and other records pertinent to the Agreement, including but not limited to records related to services rendered, quality, appropriateness, and timeliness of service, any administrative, civil or criminal investigation or prosecution. All financial records shall follow generally accepted accounting principles. Other records shall be maintained as necessary to clearly reflect all actions taken by Provider related to the Agreement. All records and reports relating to the Agreement shall be retained by Provider for a minimum of ten (10) years after final payment is made under the Agreement. However, when an inspection, audit, litigation, or other action involving records is initiated prior to the end of said period, records shall be maintained for a minimum of ten (10) years following resolution of such action. [Source: §§ 2.16.1, 2.16.2, 12.10.2 of the 2020 Apple Health IMC Agreement]

3.7 Inspection. Provider shall fully cooperate with and permit State, including HCA, MFCU and state auditor, CMS, auditors from the federal Government Accountability Office, federal Office of the Inspector General, federal Office of Management and Budget, the Office of the Inspector General, the Comptroller General, and their designees, to access, inspect and audit any books, records, contracts, or documents of Provider that pertain to any aspect of services and activities performed, including any computerized data stored by Provider, and shall permit inspection of the premises, physical facilities, and equipment where Medicaid-related activities or work is conducted, at any time whether such visit is announced or unannounced. Provider shall make staff available to assist in such inspection, review, audit, investigation, monitoring or evaluation, including the provision of adequate space on the premises to reasonably accommodate HCA, MFCU or other state or federal agency. Provider shall make copies of records and shall deliver them to the requestor, without cost, within thirty (30) calendar days of request. The right for the parties named above to audit, access and inspect under this Section exists for ten (10) years from the final date of the contract period or from the date of completion of any audit, whichever is later. If the State, CMS or the federal Office of the Inspector General determines that there is a reasonable possibility of fraud or similar risk, the State, CMS, or the federal Office of the Inspector General may inspect, evaluate, and audit the subcontractor at any time. [Source: §§ 2.14, 9.4.7, 12.10.2, 12.10.4 of the 2020 Apple Health IMC Agreement]

3.8 Interpreter Services. Provider shall provide interpreter services, free of charge, for all interactions with Covered Persons or potential Covered Persons, including but not limited to: (a) customer service, (b) all appointments with any provider for any Covered Service, (c) emergency services, and (d) all steps necessary to file grievances and appeals including requests for Independent Review of Health Plan decisions. [Source: §§ 3.3.1.1 of the 2020 Apple Health IMC Agreement]

3.9 Marketing Materials. All information to be provided to Covered Persons, e.g. marketing materials, must be accurate, not misleading, comprehensible to its intended audience, designed to provide the greatest degree of understanding, and written at a sixth (6th) grade reading level, in addition to any other requirements imposed by Health Plan based on the nature of the materials. Such materials must generally be approved by Health Plan prior to use, and must comply with the State Contract. [Source: § 3.3.2 of the 2020 Apple Health IMC Agreement]

3.10 Coordination of Benefits. Services and benefits available under the Agreement shall be secondary to any other medical coverage, except in accordance with Chapter 284-51 WAC, as applicable. Health Plan shall not refuse or reduce services provided under the Agreement solely due to the existence of similar benefits under any other health care contract, except in accord with applicable coordination of benefits rules in WAC 284-51. Health Plan shall provide prenatal care and preventive pediatric care and then seek reimbursement from third parties. Provider shall comply with all applicable Third-Party Liability and Coordination of Benefits provisions of Section 17 of the State Contract. [Source: §§ 17.2.4.1, 17.2.4.4, 17.2.4.7 of the 2020 Apple Health IMC Agreement]

3.11 Care Coordination. Provider shall comply with Health Plan's policies and support Health Plan's efforts regarding care coordination, transfers between levels of care and medication management. If applicable, Provider shall ensure that discharge plans and facilitation to post-discharge services are documented in a Covered Person's electronic health record. If Provider is an inpatient SUD treatment provider, then Provider shall have policies in place for prompt exchange of Covered Person information between behavioral health treatment agencies to facilitate continuity of care, consistent with the State Contract. [Source: § 14.17.4.3 of the 2020 Apple Health IMC Agreement]

3.12 Subcontracting. Provider may not subcontract any services under the Apple Health Program without the prior written consent of Health Plan. Any subcontract entered into by Provider must be in writing consistent with 42 C.F.R. § 434.6, and all Provider requirements contained in this Attachment must be propagated downward into any other lower tiered subcontracts. [Source: §§ 9.4, 9.4.4 of the 2020 Apple Health IMC Agreement]

3.13 Reasonable Accommodations for Disabilities. Provider shall cooperate with Health Plan to make reasonable accommodation for Covered Persons with disabilities, in accordance with the Americans with Disabilities Act, for all Covered Services and shall assure physical and communication barriers shall not inhibit Covered Persons with disabilities from obtaining Covered Services. [Source: § 2.4.40 of the 2020 Apple Health IMC Agreement]

3.14 Surgical Health and Safety. If Provider is a hospital, ambulatory care surgery center, or office-based surgery site, Provider shall endorse and adopt procedures for verifying the correct patient, the correct procedure and the correct surgical site that meet or exceed those set forth in the Universal Protocol™ development by the Joint Commission or other similar standards. [Source: § 2.25 of the 2020 Apple Health IMC Agreement]

3.15 Practice Guidelines. Provider shall comply with applicable physical and behavioral health practice guidelines adopted by Health Plan. [Source: § 7.15 of the 2020 Apple Health IMC Agreement]

3.16 Timely Access to Care. Provider shall offer access comparable to that offered to commercial enrollees or if Provider serves only Medicaid enrollees, then comparable to Medicaid fee-for-service. [Source: § 6.8.1 of the 2020 Apple Health IMC Agreement]

3.17 Hours of Operation. Provider's hours of operation for Covered Persons shall be no less than the hours of operation offered to any other of Provider's patients. [Source: § 6.6 of the 2020 Apple Health IMC Agreement]

3.18 Administrative Simplification. Unless otherwise directed by Health Plan, Provider shall use and follow the most recent updated versions of: Current Procedural Terminology ("CPT"); International Classification of Diseases ("ICD"); Healthcare Common Procedure Coding System ("HCPCS"); CMS Relative Value Units ("RVUs"); CMS billing instructions and rules; The Diagnostic and Statistical Manual of Mental Disorders; NCPDP Telecommunication Standard D.O.; and Medi-Span® Master Drug Data or any other nationally recognized drug database with approval by HCA. [Source: § 2.42.1 of the 2020 Apple Health IMC Agreement]

3.19 Claims Payment Standards. Except as otherwise allowed under Applicable Law, or unless otherwise agreed by the Parties in writing on a claim-by-claim basis, Health Plan shall meet the following minimum standards for timeliness of payment: ninety-five percent (95%) of Clean Claims shall be paid or denied within thirty (30) calendar days of receipt of the paper or electronic claim; ninety-five percent (95%) of all claims shall be paid or denied within sixty (60) calendar days of receipt of the paper or electronic claim; and ninety-nine percent (99%) of Clean Claims shall be paid or denied within ninety (90) calendar days of receipt. [Source: § 9.12.1 of the 2020 Apple Health IMC Agreement]

3.20 Appointment Wait Time Standards. As applicable, Provider shall meet the following appointment wait time standards with respect to Covered Persons:

(a) Transitional healthcare services by a PCP shall be available for clinical assessment and care planning within seven (7) calendar days of discharge from inpatient or institutional care for physical or behavioral health disorders or discharge from a Substance Use Disorder treatment program.

(b) Transitional healthcare services by a home care nurse, a home care Mental Health Professional or other Behavioral Health Professional shall be available within seven (7) calendar days of discharge from inpatient or institutional care for physical or behavioral health care, if ordered by the Covered Person's PCP or as part of the discharge plan;

(c) Preventive care office visits shall be available from the Covered Person's PCP within thirty (30) calendar days;

(d) Routine care office visits shall be available from the Covered Person's PCP within ten (10) calendar days, including behavioral health services from a behavioral health provider;

(e) Urgent, symptomatic office visits shall be available from the Covered Person's primary care, behavioral health or another provider within twenty-four (24) hours;

(f) Emergency medical care shall be available twenty-four (24) hours per day, seven (7) days per week; and

(g) Second opinion appointments described in Subsection 16.2.1 of the State Contract must occur within thirty (30) calendar days of the request, unless the Covered Person requests a postponement of the second opinion to a date later than thirty (30) calendar days.

Health Plan shall monitor Provider's compliance with this Section. In the event Provider fails to comply with the applicable appointment wait time standards set forth in this Section, Provider shall comply with Health Plan's procedures for corrective action. Nothing in this Section prohibits Provider from conducting assessments in alternate settings, such as the Covered Person's home or within an institutional setting. [Source: §§ 9.5.13, 6.9 of the 2020 Apple Health IMC Agreement]

3.21 24/7 Availability. To the extent applicable, Provider shall make the following services available twenty-four (24) hours per day, seven (7) days per week, three hundred sixty-five (365) days a year by a toll-free telephone number:

(a) Medical and behavioral health advice for Covered Persons from licensed health care professionals;

(b) Triage concerning the emergent, urgent or routine nature of medical and behavioral health conditions by licensed health care professionals; and

(c) The toll-free line staff must be able to make a warm handoff to the regional crisis line.

[Source: § 6.6 of the 2020 Apple Health IMC Agreement]

3.22 Health Information Systems. Provider shall maintain a health information system that complies with the requirements of 42 C.F.R. § 438.242 and provides the information necessary to meet Health Plan's obligations under the State Contract. Provider shall:

(a) Collect, analyze, integrate, and report data. The system must provide information on areas that include but are not limited to utilization, grievance and appeals, and terminations of enrollment for other than loss of Medicaid eligibility; and

(b) Ensure data provided to Health Plan is accurate and complete by: (a) Verifying the accuracy and timeliness of reported data; (b) Screening the data for completeness, logic, and consistency; and (c) Collecting service information on standardized formats to the extent feasible and appropriate.

[Source: § 7.16 of the 2020 Apple Health IMC Agreement]

3.23 Release of Necessary Information. Provider acknowledges and agrees to release to Health Plan any information necessary to perform any of Health Plan's obligations under the State Contract. [Source: § 9.4.6 of the 2020 Apple Health IMC Agreement]

3.24 Encounter Data Reporting. Provider shall submit complete, accurate and timely encounter data and behavioral health supplemental transactions to Health Plan in accordance with current encounter submission guidelines published by HCA or as otherwise specified by Health Plan. Provider represents and warrants that it has the capacity to submit all data required by HCA to enable Health Plan to meet the reporting requirements in the Encounter Data Reporting Guide and Behavioral Health Supplemental Transaction Data Guide published by HCA. [Source: §§ 5.14.2.2, 9.4.9 of the 2020 Apple Health IMC Agreement]

3.25 Potential Allegations of Fraud. Provider shall refer potential allegations of fraud to HCA and the Medicaid Fraud Control Unit as described in Subsection 12.6 of the State Contract. [Source: § 9.4.19 of the 2020 Apple Health IMC Agreement]

3.26 Fraud, Waste, and Abuse. Provider shall comply with Health Plan's policies regarding fraud, waste and abuse. In addition, Provider shall comply with the following applicable provisions necessary to prevent fraud, waste and abuse.

(a) If Provider is delegated responsibility for coverage of services and payment of claims under the State Contract, then Provider shall implement and maintain administrative and management arrangements or procedures designed to detect and prevent fraud, waste and abuse that are consistent with Health Plan's policies and all requirements of applicable law and the State Contract, including without limitation those described in Section 12.5.1 of the State Contract.

(b) Provider shall (i) provide written disclosure of any prohibited affiliation in accordance with 42 C.F.R. § 438.610; 42 C.F.R. § 455.106; and 42 C.F.R. § 438.608(c)(1) to HCA; (ii) provide written disclosures of information on ownership and control as indicated under Subsection 12.3 of the State contract (42 C.F.R. § 455.104; 42 C.F.R. § 455.105; and 42 C.F.R. § 438.608(c)(2)); (iii) maintain internal policies and procedures for the documentation, retention, and recovery of all overpayments, specifically for the recovery of overpayments due to fraud, waste or abuse, consistent with the State Contract; and (iv) report to Health Plan and HCA within sixty (60) calendar days when it has identified capitation payments or other payment amounts received are in excess to the amounts specified in the State Contract (42 C.F.R. § 438.608(c)(3)).

(c) Provider shall cooperate with Health Plan audits of Provider to detect and identify fraud, waste and abuse.

[Source: §§ 12.5.1, 12.5.2, 12.5.3, 12.5.4 of the 2020 Apple Health IMC Agreement]

3.27 Subrogation. Provider agrees to subrogate to the State for all criminal, civil and administrative action recoveries undertaken by any government entity, including, but not limited to, all claims Provider has or may have against any entity or individual that directly or indirectly receives funds under the State Contract including, but not limited to, any Health Care Provider, manufacturer, wholesale or retail supplier, sales representative, laboratory, or other provider in the design, manufacture, marketing, pricing, or quality of drugs, pharmaceuticals, medical supplies, medical devices, durable medical equipment, or other health care related products or services. For the purposes of this section, "subrogation" means the right of any State government entity or local law enforcement to stand in the place of Provider in the collection against a third party. [Source: § 12.6.11 of the 2020 Apple Health IMC Agreement]

3.28 Limitations on Referrals. Provider referrals may be limited to Participating Providers except in the following circumstances: (a) Emergency services; (b) Services provided outside the Service Areas as necessary to provide Medically Necessary services; (c) When a Covered Person has other primary comparable physical and/or behavioral health coverage, as necessary to coordinate benefits; and (d) Within the Service Areas, as defined in the Service Areas provisions of the Enrollment Section of the State Contract, Provider shall cover Covered Persons for all physical and/or behavioral health necessary services. [Source: § 16.1.12 of the 2020 Apple Health IMC Agreement]

3.29 High Categorical Risk Providers. Providers that are deemed to be “high categorical risk,” including prospective (newly enrolling) home health agencies and prospective (newly enrolling) durable medical equipment, prosthetics, orthotics and supplies (DMEPOS) suppliers or such other categories of providers as defined under 42 C.F.R. § 424.518, shall be enrolled in and screened by Medicare, in addition to complying with Health Plan’s policies and procedures regarding credentialing and recredentialing. Such providers shall revalidate Medicare enrollment every five (5) years in compliance with 42 C.F.R. § 424.515. Notwithstanding the foregoing, Infant In-Home Phototherapy Providers that meet Health Plan’s certification requirements are not required to be enrolled in Medicare. [Source: § 9.15.13 of the 2020 Apple Health IMC Agreement]

3.30 HCA Approval Required. To the extent that the Agreement is considered a “Subcontract” requiring HCA approval under the State Contract, the Agreement will not take effect prior to HCA’s review and written approval, or failure to approve or deny within forty-five (45) calendar days of filing. [Source: § 9.1.3 of the 2020 Apple Health IMC Agreement]

3.31 HCA Approval for Assignment. Provider acknowledges and agrees that no assignment of the Agreement shall take effect without the prior written agreement of HCA. [Source: § 2.2 of the 2020 Apple Health IMC Agreement]

3.32 Quality Improvement System. Provider shall maintain a quality improvement system tailored to the nature and type of Covered Services provided hereunder, which affords quality control for such services, including but not limited to the accessibility of Medically Necessary services, and which provides for a free exchange of information with Health Plan to assist Health Plan in complying with the requirements of the State Contract. Providers that are PCPs or specialty care providers shall comply with all quality improvement activities of the Health Plan. [Source: §§ 9.5.1, 9.5.2 of the 2020 Apple Health IMC Agreement]

3.33 Records of Delegated Activities. As applicable to services rendered under the Agreement, Provider shall have a means to keep records necessary to adequately document services provided to Covered Persons for any and all delegated activities including quality improvement, utilization management, Covered Person’s rights and responsibilities, Health Homes, and credentialing and re-credentialing. [Source: § 9.5.3 of the 2020 Apple Health IMC Agreement]

3.34 Behavioral Health Provider Supervision. Provider agrees that, if applicable, it will receive payment for the supervision of behavioral health providers whose license or certification restricts them to working under supervision. [Source: §§ 9.5.7 of the 2020 Apple Health IMC Agreement]

3.35 Payment in Full and Covered Person Charges. Provider agrees to accept payment from Health Plan as payment in full. Provider shall not request payment from HCA or any Covered Person for Covered Services provided under the Agreement, and shall comply with WAC 182-502-0160 requirements applicable to providers. Provider shall report to Health Plan any instance in which a Covered Person is charged for services. Provider shall repay to a Covered Person any inappropriate charges paid by such Covered Person, or shall reimburse Health Plan to the extent Health Plan repays such inappropriate charges to the Covered Person. [Source: §§ 9.5.8, 10.6.3 of the 2020 Apple Health IMC Agreement]

3.36 HCA and Covered Person Hold Harmless. Provider agrees to hold harmless HCA and its employees, and all Covered Persons in the event of non-payment by Health Plan. Provider further agrees to indemnify and hold harmless HCA and its employees against (a) all injuries, deaths, losses, damages, claims, suits, liabilities, judgments,

costs and expenses which may in any manner accrue against HCA or its employees through the intentional misconduct, negligence, or omission of Provider, its agents, officers, employees or contractors, and (b) any damages related to Provider's unauthorized use or release of Personal Information or PHI of Covered Persons. [Source: § 9.5.9 of the 2020 Apple Health IMC Agreement]

3.37 Termination Provision. Either Party to this Attachment may terminate this Attachment upon ninety (90) days advance written notice to the other Party. Notwithstanding the foregoing, in the event that (a) Provider is excluded from participation in the Medicaid program, Health Plan may immediately terminate the Agreement or this Attachment upon written notice to Provider, and may immediately recover any payments for goods or services that benefit excluded individuals or entities; or (b) HCA or Medicare has taken any action to revoke Provider's privileges for cause, and Provider has exhausted all applicable appeal rights or the timeline for appeal has expired. "For cause" may include but is not limited to reasons related to fraud, integrity or quality. [Source: §§ 9.5.12, 9.5.13, 12.8.2, 9.15.14 of the 2020 Apple Health IMC Agreement]

3.38 Provider Appeal Rights. If Provider provides physician services, Provider may exercise any appeal rights pursuant to Chapters 284-43 and 284-170 WAC to challenge Health Plan's failure to cover a service. [Source: § 9.5.11 of the 2020 Apple Health IMC Agreement]

3.39 Health Plan Oversight and Corrective Action. Provider acknowledges and agrees that Health Plan shall conduct ongoing monitoring and periodic formal review that is consistent with applicable industry standards and the regulations of the Washington State Office of the Insurance Commissioner, if any. Such formal review shall be completed no less than once every three (3) years or more often if specified, and will identify any deficiencies or areas of improvement and provide for corrective action of any such deficiencies. Such review shall include an evaluation to ensure that services furnished by Provider to individuals with special health care needs are appropriate to the Covered Person's needs. Inadequate performance under the Agreement will be subject to the revocation of delegation or imposition of sanctions in accordance with the dispute resolution process detailed in the Agreement. [Source: §§ 9.4.2, 9.4.15, 9.4.16, 9.5.16, 9.6.1.4 of the 2020 Apple Health IMC Agreement]

3.40 Covered Person Self-Referral. Provider acknowledges that Covered Persons have a right to self-refer for:

(a) Family planning services and supplies, and sexually-transmitted disease screening and treatment services provided at participating or non-Participating Providers, including but not limited to family planning agencies;

(b) Immunizations, sexually-transmitted disease screening and follow-up, immunodeficiency virus (HIV) screening, tuberculosis screening and follow-up, and family planning services through and if provided by a local health department;

(c) Immunizations, sexually transmitted disease screening, family planning and behavioral health services through and if provided by a school-based health center;

(d) All services received by American Indian or Alaska Native Covered Persons under the Special Provisions for American Indians and Alaska Natives Subsection of the State Contract; and

(e) Crisis Response Services, including crisis intervention; crisis respite; investigation and detention services; and, evaluation and treatment services. Self-referrals can also be made for assessment and intake for behavioral health services.

[Source: § 16.11.7 of the 2020 Apple Health IMC Agreement]

3.41 Delegated Administrative Services Agreement. In the event that the Agreement delegates administrative functions to Provider, including Essential Behavioral Health Administrative Functions, the Parties

agree that they shall enter into a delegated administrative services agreement that contains all provisions required pursuant to the State Contract. [Source: §§ 9.6, 9.7 of the 2020 Apple Health IMC Agreement]

3.42 Confidential Member Information. Provider shall keep information about Covered Persons, including their medical records, confidential in a manner consistent with Applicable Law. [Source: § 9.5.6 of the 2020 Apple Health IMC Agreement]

3.43 Medicaid NCCI Files. Provider may disclose only non-confidential information that is also available to the general public about the Medicaid NCCI edit files on the Medicaid NCCI webpage. [Source: § 2.42.3.2 of the 2020 Apple Health IMC Agreement]

3.44 Member Rights. Provider shall comply with any Applicable Law that pertain to Covered Persons' rights and shall protect and promote those rights when furnishing services to Covered Persons. Provider shall guarantee each Covered Person the rights set forth below. Each Covered Person must be free to exercise these rights and the exercise of these rights must not adversely affect the way Health Plan or Provider treats the Covered Person. These rights include:

- (a) To be treated with respect and with consideration for Covered Person's dignity and privacy;
- (b) To receive information on available treatment options and alternatives, presented in a manner appropriate to the Covered Person's ability to understand;
- (c) To participate in decisions regarding Covered Person's health care, including the right to refuse treatment;
- (d) To be free from any form of restraint or seclusion used as a means of coercion, discipline, convenience, or retaliation;
- (e) To request and receive a copy of their medical records, and to request that they be amended or corrected in accordance with Applicable Law; and
- (f) To choose a behavioral Health Care Provider.

[Source: §§ 10.1.1, 10.1.2 of the 2020 Apple Health IMC Agreement]

3.45 Background Checks. Provider shall require a criminal history background check through the Washington State Patrol for employees and volunteers of Provider who may have unsupervised access to children, people with developmental disabilities, or vulnerable adults. Further, Provider shall maintain related policies and procedures and personnel files consistent with requirements in Chapter 43.43 RCW, Chapters 388-877 WAC and Chapter 388-06A WAC. [Source: § 10.1.3 of the 2020 Apple Health IMC Agreement]

3.46 Cultural Considerations. If applicable, Provider shall participate in and cooperate with Health Plan's efforts to promote the delivery of services in a culturally competent manner to all Covered Persons, including those with limited English proficiency and diverse cultural and ethnic backgrounds, disabilities, and regardless of gender, sexual orientation or gender identity. [Source: § 10.2 of the 2020 Apple Health IMC Agreement]

3.47 Member Self-Determination. Provider shall (a) obtain informed consent prior to treatment from all Covered Persons, or from persons authorized to consent on behalf of Covered Persons as described in RCW 7.70.065, (b) comply with the provisions of the Natural Death Act (Chapter 70.122 RCW) and Applicable Law and rules concerning advance directives and POLST (eg., WAC 182-501-0125 and 42 C.F.R. § 417.436), and (c) when appropriate, inform Covered Persons of their right to make anatomical gifts pursuant to Chapter 68.64 RCW. [Source: § 10.8 of the 2020 Apple Health IMC Agreement]

3.48 Advance Directives and POLST. Provider shall ensure that whether a Covered Person has executed an advance directive or POLST shall be indicated in a prominent part of such Covered Person's medical records, and Provider shall not provision care or otherwise discriminate against a Covered Person based on whether the Covered Person has executed an advance directive or POLST. [Source: §§ 10.3.5, 10.3.6 of the 2020 Apple Health IMC Agreement]

3.49 Mental Health Advance Directives. Provider shall comply with Chapter 71.32 RCW (Mental Health Advance Directives). [Source: § 9.5.4 of the 2020 Apple Health IMC Agreement]

3.50 Insurance. Provider shall have and maintain insurance appropriate to the services to be performed under the Agreement. Provider shall make copies of certificates of insurance available to HCA upon request. [Source: § 2.15.5 of the 2020 Apple Health IMC Agreement]

3.51 Health Home Surety Bond. If Provider is a home health agency, Provider represents and warrants that it is in compliance with the surety bond requirements of federal law (Section 4708(d) of the Balanced Budget Act of 1997 and 42 C.F.R. § 441.16). [Source: § 9.9 of the 2020 Apple Health IMC Agreement]

3.52 Solvency Requirements. If Provider is at financial risk, as defined in the Substantial Financial Risk or Risk provisions in the State Contract, Provider shall be subject to solvency requirements that provide assurance of Provider's ability to meet its obligations. Such requirements shall be regularly monitored and enforced. [Source: § 9.2 of the 2020 Apple Health IMC Agreement]

3.53 Physician Incentive Plans. If Provider makes payment to any physician under a Physician Incentive Plan, such plan shall meet all applicable requirements under the State Contract, including but not limited to disclosure requirements and stop-loss protection. No payment to Provider, or by Provider to a provider, under a Physician Incentive Plan shall, directly or indirectly, be an inducement to reduce or limit Medically Necessary Services provided to an individual Covered Person. [Source: § 9.10 of the 2020 Apple Health IMC Agreement]

3.54 Information on Ownership and Control.

(a) Provider shall disclose the following information to Health Plan upon Agreement execution, upon request during the re-validation of enrollment process under 42 C.F.R. § 455.414, and within thirty-five (35) business days after any change in ownership of Provider:

(1) The name and address of any person (individual or corporation) with an ownership or control interest in Provider;

(2) If Provider is a corporate entity, the primary business address, every business location, and P.O. Box address;

(3) If Provider has corporate ownership, the tax identification number of the corporate owner(s);

(4) If Provider is an individual, date of birth and Social Security Number;

(5) If Provider has a five percent (5%) ownership interest in any of its subcontractors, the tax identification number of the subcontractor(s);

(6) Whether any person with an ownership or control interest in Provider is related by marriage or blood as a spouse, parent, child, or sibling to any other person with an ownership or control interest in Provider;

(7) If Provider has a five percent (5%) ownership interest in any of its subcontractors, whether any person with an ownership or control interest in such subcontractor is related by marriage or blood as a spouse, parent, child, or sibling to any other person with an ownership or control interest in Provider; and

(8) Whether any person with an ownership or control interest in Provider also has an ownership or control interest in any Managed Care entity.

[Source: § 12.3.1 of the 2020 Apple Health IMC Agreement]

(b) Upon the request of Health Plan or HCA, Provider shall furnish to HCA, within thirty-five (35) calendar days of a request, full and complete business transaction information as follows:

(1) The ownership of any subcontractor with whom Provider has had business transactions totaling more than twenty-five thousand dollars (\$25,000.00) during the previous twelve (12) month period ending on the date of the request; and

(2) Any significant business transaction between Provider and any wholly owned supplier or any subcontractor during the previous five (5) year period ending on the date of the request.

Provider shall provide any further information needed or reasonably requested by Health Plan for the purpose of satisfying Health Plan's HCA reporting requirements under the State Contract, or for the purpose of verifying or screening for exclusion from federal or state health care programs, or for conviction of various criminal or civil offences, among the individuals or entities who have an ownership or control interest in, or who are a managing employee of, Provider.

[Source: § 12.3.2, 12.3.1 of the 2020 Apple Health IMC Agreement]

(c) Upon request, Provider shall furnish to the Washington Secretary of State, the Secretary of the US Department of Health and Human Services, the Inspector General of the US Department of Health and Human Services, the Washington State Auditor, the Comptroller of the Currency, and HCA a description of the transaction between Provider and a party in interest (as defined in Section 1318(b) of the Public Health Service Act) within thirty-five (35) calendar days of the request, including the following transactions:

(1) Any sale or exchange, or leasing of any property between Provider and such a party;

(2) Any furnishing for consideration of goods, services (including management services), or facilities between Provider and such a party but not including salaries paid to employees for services provided in the normal course of their employment; and

(3) Any lending of money or other extension of credit between Provider and such a party.

[Source: § 12.3.3 of the 2020 Apple Health IMC Agreement]

3.55 Information on Persons Convicted of Crimes. If the Provider is not an individual practitioner or a group of practitioners, Provider shall investigate and disclose to Health Plan, at Agreement execution or renewal, and upon request by Health Plan of the identified person who has been convicted of a criminal offense related to that person's involvement in any program under Medicare, Medicaid, or the title XX services program since the inception of those programs and who is:

(a) A person who has an ownership or control interest in Provider;

(b) An agent or person who has been delegated the authority to obligate or act on behalf of Provider; and

(c) An agent, managing employee, general manager, business manager, administrator, director, or other individual who exercises operational or managerial control over, or who directly or indirectly conducts the day-to-day operation of, Provider.

[Source: § 12.4 of the 2020 Apple Health IMC Agreement]

3.56 Maternity Newborn Length of Stay; Sterilizations and Hysterectomies. All hospital delivery maternity care provided under the Agreement shall be in accord with RCW 48.43.115. All sterilizations and hysterectomies provided under the Agreement shall be in compliance with 42 C.F.R. § 441 Subpart F, and Provider shall use a Consent for Sterilization form (HHS-687) or its equivalent in connection therewith. A hysterectomy requires the Hysterectomy Consent and Patient Information form (HCA 13-365). [Source: §§ 10.10, 16.3 of the 2020 Apple Health IMC Agreement]

3.57 Grievance and Appeals. Health Plan shall maintain a grievance and appeals system in accordance with the requirements of the State Contract, and Health Plan shall provide the following information regarding Health Plan's grievance and appeal system to Provider:

- (a) The toll-free numbers to file oral grievances and appeals;
- (b) The availability of assistance in filing a grievance or appeal;
- (c) The Covered Person's right to request continuation of Medicaid benefits during an appeal or hearing and, if the Health Plan's Adverse Benefit Determination is upheld, that the Covered Person may be responsible to pay for the continued benefits;
- (d) The Covered Person's right to file grievances and appeals and their requirements and timeframes for filing;
- (e) The Covered Person's right to a hearing, how to obtain a hearing and representation rules at a hearing; and
- (f) Provider may file a grievance or request an adjudicative proceeding on behalf of a Covered Person in accordance with the State Contract.

[Source: § 9.4.14 of the 2020 Apple Health IMC Agreement]

3.58 Integrated Patient Record/Clinical Data Repository. If Provider has a certified EHR system, Provider shall submit automated exports of standard CCD/CCDA, or subsequent ONC-Specified standard healthcare transactions, as specified by HCA and consistent with the design of the integrated patient record, from their EHR to the CDR via the HIE each time a Covered Person is seen. [Source: § 7.3.8 of the 2020 Apple Health IMC Agreement]

3.59 Training. If Provider is a behavioral health network provider, Provider must participate in training when requested by HCA, unless Provider receives an exception from the HCA after submitting to the HCA a written exception request along with a plan for how the required information will be provided to the Provider's staff. [Source: § 9.11.2.2.4 of the 2020 Apple Health IMC Agreement]

3.60 WISe Service Providers. If Provider provides WISe services, Provider shall adhere to the most current version of the WISe Manual and participate in all WISe-related activities. Without limiting the generality of the foregoing, Provider shall participate in a review of WISe services conducted using the WISe Quality Improvement Review Tool (QIRT) at least once during the contract period. [Source: § 14.1.8.1 of the 2020 Apple Health IMC Agreement]

3.61 GAIN-SS. Provider shall use GAIN-SS and assessment process that includes use of the quadrant placement. Provider shall be subject to corrective action if the Integrated Co-Occurring Disorder Screening and Assessment process is not implemented and maintained throughout the term of the Agreement. [Source: § 9.7.3 of the 2020 Apple Health IMC Agreement]

3.62 Disaster Recovery Plan. If applicable, Provider shall develop and maintain a business continuity and disaster recovery plan that ensures timely reestablishment of the Covered Person information system following total loss of the primary system or a substantial loss of functionalist. Provider's plan shall comply with all relevant requirements in the State Contract and Provider shall cooperate with Health Plan to comply with all related reporting requirements. [Source: § 18.2.1.1 of the 2020 Apple Health IMC Agreement]

3.63 Indian Health Care Provider. If Provider is an Indian Health Care Provider, then it shall be subject to the "Medicaid and CHIP Managed Care Addendum for IHCPs", attached hereto as ATTACHMENT D. [Source: § 9.5.21 of the 2020 Apple Health IMC Agreement]

**ATTACHMENT A: Medicaid
Network: CCCWA**

**EXHIBIT A-3
APPLE HEALTH
COMPENSATION SCHEDULE
PRACTITIONER SERVICES**

RadiantCare Physicians PLLC

This Compensation Schedule sets forth the maximum reimbursement amounts for the provision of Covered Services to Covered Persons in a Medicaid Product offered through Health Plan and referred to as Apple Health. For Covered Services rendered to a Covered Person and billed under a Contracted Provider's tax identification number ("TIN") that has been designated by the Payor as subject to this Compensation Schedule, Payor shall pay or arrange for payment of a Clean Claim for Covered Services rendered by the Contracted Provider according to the terms of the Agreement and this Compensation Schedule. Payment under this Compensation Schedule is subject to the requirements set forth in the Agreement.

For Behavioral Health Agency Covered Services provided to Covered Persons, Contracted Provider's maximum compensation shall be the Allowed Amount. Except as otherwise provided in this Compensation Schedule the Allowed Amount is the lesser of: (i) the Contracted Provider's Allowable Charges; or (ii) one hundred percent (100%) of the Coordinated Care Behavioral Health Agency fee schedule in effect on the date of service.

For Covered Services not covered above provided to Covered Persons, Contracted Provider's maximum compensation shall be the Allowed Amount. Except as otherwise provided in this Compensation Schedule the Allowed Amount is the lesser of: (i) the Contracted Provider's Allowable Charges; or (ii) one hundred percent (100%) of the State's Medicaid fee schedule in effect on the date of service.

If there is no established payment amount on the State's Medicaid fee schedule for a Covered Service provided to a Covered Person, Payor may establish a payment amount to apply in determining the Allowed Amount. Until such time as Payor establishes such a payment amount, Contracted Provider's Allowed Amount shall be twenty-five percent (25%) of the Contracted Provider's Allowable Charge.

Additional Provisions:

1. **Code Change Updates.** Payor utilizes nationally recognized coding structures (including, without limitation, revenue codes, CPT codes, HCPCS codes, ICD codes, national drug codes, ASA relative values, etc., or their successors) for basic coding and descriptions of the services rendered. Updates to billing-related codes shall become effective on the date ("Code Change Effective Date") that is the later of: (i) the first day of the month following sixty (60) days after publication by the governmental agency having authority over the applicable product of such governmental agency's acceptance of such code updates, (ii) the effective date of such code updates as determined by such governmental agency or (iii) if a date is not established by such governmental agency or the product is not regulated by such governmental agency, the date that changes are made to nationally recognized codes. Such updates may include changes to service groupings. Claims processed prior to the Code Change Effective Date shall not be reprocessed to reflect any such code updates.
2. **Fee Change Updates.** Updates to the fee schedule shall become effective on the effective date of such fee schedule updates, as determined by the Payor ("Fee Change Effective Date"). However, the date of implementation of any fee schedule updates, i.e. the date beginning on which such fee change is used for reimbursement ("Fee Change Implementation Date") shall be the later of: (i) the date on which Payor is reasonably able to implement the update in the claims payment system; or (ii) the Fee Change Effective Date. Claims processed prior to the Fee Change Implementation Date shall not be reprocessed to reflect any updates to such fee schedule, even if service was provided after the Code Change Effective Date.

Definitions:

1. **Allowed Amount** means the amount designated as the maximum amount payable to a Contracted Provider for any particular Covered Service provided to any particular Covered Person, pursuant to this Agreement or its Attachments for Covered Services.
2. **Allowable Charges** means a Contracted Provider's billed charges for services that qualify as Covered Services.

Attachment C: Commercial-Exchange

EXHIBIT C-0 AMBETTER PARTICIPATING PROVIDER AGREEMENT ATTACHMENT

This COMMERCIAL-EXCHANGE PRODUCT - PARTICIPATING PROVIDER AGREEMENT ATTACHMENT - (this “**Product Attachment**”) is incorporated into the Participating Provider Agreement (the “**Agreement**”) entered into by and between Health Plan and Provider (as such entities are defined in the Agreement).

RECITALS

WHEREAS, Health Plan and Provider entered into the Agreement, as the same may have been amended and supplemented from time to time, pursuant to which Provider and its Contracted Providers or other Downstream Entities participate in certain Products offered by or available from or through a Company; and

WHEREAS, pursuant to the provisions of the Agreement, this Product Attachment is identified on Schedule B of the Agreement and, as such, the Contracted Providers identified herein will be designated and participate as Participating Providers in the Product described in this Product Attachment, and will be considered to be and will be governed under this Product Attachment as Downstream Entities, as defined in this Product Attachment; and

WHEREAS, the Agreement is modified or supplemented as hereafter provided.

NOW THEREFORE, in consideration of the recitals, the mutual promises herein stated, the parties hereby agree to the provisions set forth below.

TERMS

1. Defined Terms. For purposes of the Commercial-Exchange Product, the following terms have the meanings set forth below. All capitalized terms not specifically defined in this Product Attachment will have the meanings given to such terms in the Agreement.

1.1 “**Commercial-Exchange Product**”, also referred to as “**Ambetter**”, refers to those programs and health benefit arrangements offered by a Company that provide incentives to Covered Persons to utilize the services of certain contracted providers. The Commercial-Exchange Product includes those Coverage Agreements entered into, issued or agreed to by a Payor under which a Company furnishes administrative services or other services in support of a health care program for an individual or group of individuals, which may include access to one or more of the Company’s provider networks or vendor arrangements, and which may be provided in connection with a state or governmental-sponsored, employer-sponsored or other private health insurance exchange, except those excluded by Health Plan. The Commercial-Exchange Product does not apply to any Coverage Agreements that are specifically covered by another Product Attachment to the Agreement.

1.2 “**Delegated Entity**” means any party, including an agent or broker, that enters into an agreement with Health Plan to provide administrative services or health care services to qualified individuals, qualified employers or qualified employees and their dependents (as such terms are defined in 45 C.F.R. §156.20).

1.3 “**Downstream Entity**” means any party, including an agent or broker, that enters into an agreement with a Delegated Entity or with another Downstream Entity for purposes of providing administrative or health care services related to the agreement between the Delegated Entity and Health Plan. The term “Downstream Entity” is intended to reach the entity that directly provides administrative services or health care services to qualified individuals, qualified employers, or qualified employees and their dependents (as such terms are defined in 45 C.F.R. §156.20).

1.4 “**Emergency**” or “**Emergency Care**” has the meaning set forth in the Covered Person’s Coverage Agreement.

1.5 “**Emergency Medical Condition**” has the meaning set forth in the Covered Person’s Coverage Agreement.

1.6 “**State**” means the State of Washington, or such other state to the extent that a Coverage Agreement or Covered Person is subject to such other state’s law.

2. Commercial-Exchange Product. This Product Attachment constitutes the “Commercial-Exchange Product (Ambetter) Attachment” and is incorporated into the Agreement. It supplements the Agreement by setting forth specific terms and conditions that apply to the Commercial-Exchange Product with respect to which a Participating Provider has agreed to participate, and with which a Participating Provider must comply in order to maintain such participation. This Product Attachment applies with respect to the provision of health care services, supplies or accommodations (including Covered Services) to Covered Persons enrolled in or covered by a Commercial-Exchange Product.

3. Participation. Except as otherwise provided in this Product Attachment or the Agreement, all Contracted Providers under the Agreement will participate as Participating Providers in this Commercial-Exchange Product, and will provide to Covered Persons enrolled in or covered by a Commercial-Exchange Product, upon the same terms and conditions contained in the Agreement, as supplemented or modified by this Product Attachment, those Covered Services that are provided by Contracted Providers pursuant to the Agreement. In providing such services, Provider shall, and shall cause Contracted Providers, to comply with and abide by the provisions of this Product Attachment and the Agreement (including the Company’s policies and procedures).

3.1 Network Name. All Contracted Providers under the Agreement will participate as Participating Providers in and become part of the **CCCWA Exchange** network.

4. Attachments. This Product Attachment includes the Compensation Schedules for the Commercial-Exchange Product, as indicated on Schedule B of the Agreement, each of which are incorporated herein by reference.

5. Construction. This Product Attachment supplements and forms a part of the Agreement. Except as otherwise provided herein or in the terms of the Agreement, the terms and conditions of the Agreement will remain unchanged and in full force and effect as a result of this Product Attachment. In the event of a conflict between the provisions of the Agreement and the provisions of this Product Attachment, this Product Attachment will govern with respect to health care services, supplies or accommodations (including Covered Services) rendered to Covered Persons enrolled in or covered by a Commercial-Exchange Product. To the extent Provider or any Contracted Provider is unclear about its, his or her respective duties and obligations, Provider or the applicable Contracted Provider shall request clarification from the Company.

6. Term. This Product Attachment will become effective as of the Effective Date, and will be coterminous with the Agreement unless a Party terminates the participation of the Contracted Provider in this Commercial-Exchange Product in accordance with the applicable provisions of the Agreement or this Product Attachment.

7. Federal Requirements. The following requirements apply to Delegated and Downstream Entities under this Product Attachment, which includes but is not limited to Provider and all Contracted Providers.

7.1 Provider’s delegated activities and reporting responsibilities, if any, are specified in the Agreement or applicable attachment to the Agreement (e.g., Delegated Credentialing Agreement, Delegated Services Agreement, Statement of Work, or other scope of services attachment) attached to this Agreement. If such attachment is not executed, no administrative functions shall be deemed as delegated.

7.2 CMS, Health Plan and Payor reserve the right to revoke the delegation activities and reporting requirements or to specify other remedies in instances where CMS, Health Plan or the Payor determine that Provider or any Downstream Entity has not performed satisfactorily.

7.3 Provider and all Downstream Entities must comply with all applicable laws and regulations relating to the standards specified under 45 CFR §156.340(a);

7.4 Provider and all Downstream Entities must permit access by the Secretary and OIG or their designees in connection with their right to evaluate through audit, inspection or other means, to the Provider's or Downstream Entities' books, contracts, computers, or any other electronic systems including medical records and documentation, relating to Health Plan's obligations in accordance with federal standards under 45 CFR §156.340(a) until ten (10) years from the termination date of this Product Attachment.

8. Other Terms and Conditions. Except as modified or supplemented by this Product Attachment, the compensation hereunder for the provision of Covered Services by Contracted Providers to Covered Persons enrolled in or covered by the Commercial-Exchange Product is subject to all of the other provisions in the Agreement (including the Company's policies and procedures) that affect or relate to compensation for Covered Services provided to Covered Persons.

ATTACHMENT C: Commercial-Exchange

Network: CCCWA Exchange

EXHIBIT C-2 AMBETTER COMPENSATION SCHEDULE PRACTITIONER SERVICES

RadiantCare Physicians PLLC

This Compensation Schedule sets forth the maximum reimbursement amounts for the provision of Covered Services to Covered Persons in a Commercial-Exchange Product offered through Health Plan and referred to as Ambetter. For Covered Services rendered to a Covered Person and billed under a Contracted Provider's tax identification number ("TIN") that has been designated by the Payor as subject to this Compensation Schedule, Payor shall pay or arrange for payment of a Clean Claim for Covered Services rendered by the Contracted Provider according to the terms of the Agreement and this Compensation Schedule. Payment under this Compensation Schedule is subject to the requirements set forth in the Agreement, which include reducing the Allowed Amount by the applicable Cost-Sharing Amounts.

For Practitioner Covered Services provided to Covered Persons, Contracted Provider's maximum compensation shall be the Allowed Amount. Except as otherwise provided in this Compensation Schedule, the Allowed Amount is the lesser of: (i) the Contracted Provider's Allowable Charges; or (ii) one hundred percent (100%) of the Payor's Fee Schedule.

Multiple Procedure Pricing Rules. Multiple procedures performed during the same day will be reimbursed at one hundred percent (100%) for the primary procedure, fifty percent (50%) for the second procedure, and fifty percent (50%) for the third procedure, subsequent procedures shall not be eligible for reimbursement.

Additional Provisions:

1. **Code Change Updates.** Payor utilizes nationally recognized coding structures (including, without limitation, revenue codes, CPT codes, HCPCS codes, ICD codes, national drug codes, ASA relative values, etc., or their successors) for basic coding and descriptions of the services rendered. Updates to billing-related codes shall become effective on the date ("Code Change Effective Date") that is the later of: (i) the first day of the month following sixty (60) days after publication by the governmental agency having authority over the applicable product of such governmental agency's acceptance of such code updates, (ii) the effective date of such code updates as determined by such governmental agency or (iii) if a date is not established by such governmental agency or the product is not regulated by such governmental agency, the date that changes are made to nationally recognized codes. Such updates may include changes to service groupings. Claims processed prior to the Code Change Effective Date shall not be reprocessed to reflect any such code updates.
2. **Fee Change Updates.** Updates to the fee schedule shall become effective on the effective date of such fee schedule updates, as determined by the Payor ("Fee Change Effective Date"). However, the date of implementation of any fee schedule updates, i.e. the date beginning on which such fee change is used for reimbursement ("Fee Change Implementation Date") shall be the later of: (i) the date on which Payor is reasonably able to implement the update in the claims payment system; or (ii) the Fee Change Effective Date. Claims processed prior to the Fee Change Implementation Date shall not be reprocessed to reflect any updates to such fee schedule, even if service was provided after the Code Change Effective Date.
3. **Modifier.** Unless specifically indicated otherwise, the Allowed Amount represents global fees and may be subject to reductions based on appropriate modifiers (for example, professional and technical modifiers). As used in the previous sentence, "global fees" refers to services billed without a modifier, for which the Allowed Amount includes both the professional component and the technical component.

4. Anesthesia Modifier Pricing Rules. The dollar amounts that will be used in the calculation of Anesthesia Management fees are in accordance with the Anesthesia Payment Policy. Unless specifically stated otherwise, the Anesthesia Conversion Factor indicated is fixed and will not change. The Anesthesia Conversion Factor is based on an anesthesia time unit value of 15 minutes.
5. Place of Service Pricing Rules. Payor will follow CMS guidelines for determining when services are priced at the facility or non-facility fee schedule (with the exception of services performed at Ambulatory Surgery Centers, POS 24, which will be priced at the facility fee schedule).

Definitions:

1. **Allowed Amount** means the amount designated as the maximum amount payable to a Contracted Provider for any particular Covered Service provided to any particular Covered Person, pursuant to this Agreement or its Attachments for Covered Services.
2. **Allowable Charges** means a Contracted Provider's billed charges for services that qualify as Covered Services.
3. **Cost-Sharing Amounts** means any amounts payable by a Covered Person, such as copayments, cost-sharing, coinsurance, deductibles or other amounts that are the Covered Person's financial responsibility under the applicable Coverage Agreement, if applicable.

Attachment G: Cascade Care Public Option

EXHIBIT G-0 CASCADE CARE PUBLIC OPTION PARTICIPATING PROVIDER AGREEMENT ATTACHMENT

This CASCADE CARE PRODUCT - PARTICIPATING PROVIDER AGREEMENT ATTACHMENT - (this “**Product Attachment**”) is incorporated into the Participating Provider Agreement (the “**Agreement**”) entered into by and between Health Plan and Provider (as such entities are defined in the Agreement).

RECITALS

WHEREAS, Health Plan and Provider entered into the Agreement, as the same may have been amended and supplemented from time to time, pursuant to which Provider and its Contracted Providers or other Downstream Entities participate in certain Products offered by or available from or through Health Plan; and

WHEREAS, Health Plan has contracted with HCA to arrange for the provision of health care service to Covered Persons enrolled in a Public Option Plan, and

WHEREAS, this Product Attachment is intended to supplement the Agreement by setting for the parties’ rights and responsibilities related to the provision of Covered Services as it pertains to the Cascade Care Public Option Product; and

WHEREAS, pursuant to the provisions of the Agreement, this Product Attachment is identified on Schedule B of the Agreement and, as such, the Contracted Providers identified herein will be designated and participate as Participating Providers in the Product described in this Product Attachment, and will be considered to be and will be governed under this Product Attachment as Downstream Entities, as defined in this Product Attachment; and

WHEREAS, the Agreement is modified or supplemented as hereafter provided.

NOW THEREFORE, in consideration of the recitals, the mutual promises herein stated, the parties hereby agree to the provisions set forth below.

TERMS

Defined Terms. For purposes of the Cascade Care Product, the following terms have the meanings set forth below. All capitalized terms not specifically defined in this Product Attachment will have the meanings given to such terms in the Agreement.

1.1 “**Breach**” means the unauthorized acquisition, access, use, or disclosure of Confidential Information that compromises the security, confidentiality, or integrity of the Confidential Information.

1.2 “**Cascade Care Product**”, also referred to as “Cascade Care”, refers the Public Option Plan offered by Health Plan that provide incentives to Covered Persons to utilize the services of certain contracted providers and which are subject to the terms and conditions specified in Engrossed Substitute Senate Bill 5526 (2019) and the rules promulgated thereunder, as they may be amended or repealed.

1.3 “**Carrier**” means a disability insurer regulated under chapter 48.20 or 48.21 RCW, a health care service contractor as defined in RCW 48.44.010, or a health maintenance organization as defined in RCW 48.46.020.

1.4 “**Confidential Information**” means information that may be exempt from disclosure to the public or other unauthorized persons under chapter 42.56 RCW or chapter 70.02 RCW or other state or federal

statutes or regulations. Confidential Information includes, but is not limited to, any information identifiable to an individual that relates to a natural person's health (see also Protected Health Information), finances, education, business, use or receipt of governmental services, names, addresses, telephone numbers, social security numbers, driver license numbers, financial profiles, credit card numbers, financial identifiers and any other identifying numbers, law enforcement records, HCA source code or object code, or HCA or State security information.

1.5 ***"Delegated Entity"*** means any party, including an agent or broker, that enters into an agreement with Health Plan to provide administrative services or health care services to qualified individuals, qualified employers or qualified employees and their dependents (as such terms are defined in 45 C.F.R. §156.20).

1.6 ***"Downstream Entity"*** means any party, including an agent or broker, that enters into an agreement with a Delegated Entity or with another Downstream Entity for purposes of providing administrative or health care services related to the agreement between the Delegated Entity and Health Plan. The term "Downstream Entity" is intended to reach the entity that directly provides administrative services or health care services to qualified individuals, qualified employers, or qualified employees and their dependents (as such terms are defined in 45 C.F.R. §156.20).

1.7 ***"HCA"*** means the State of Washington Health Care Authority and its employees and authorized agents.

1.8 ***"Medicare"*** means the health insurance program for the aged and disabled under title XVIII of the Social Security Act.

1.9 ***"PO HCA Contract"*** means the Contract for Cascade Care Public Option Plans on the Washington Healthplanfinder entered into by and between HCA and Health Plan.

1.10 ***"Protected Health Information"*** means individually identifiable information that relates to the provision of health care to an individual; the past, present, or future physical or mental health or condition of an individual; or past, present, or future payment for provision of health care to an individual, as defined in 45 CFR 160.103. Individually identifiable information is information that identifies the individual or about which there is a reasonable basis to believe it can be used to identify the individual, and includes demographic information. PHI is information transmitted, maintained, or stored in any form or medium. 45 CFR 164.501. PHI does not include education records covered by the Family Educational Rights and Privacy Act, as amended, 20 USC 1232g(a)(4)(b)(iv).

1.11 ***"Public Option Participating Provider"*** means Participating Provider who provides Covered Services to Covered Persons pursuant to Health Plan's Public Option Plan.

1.12 ***"Public Option Plan"*** means a qualified health plan procured by HCA and offered on the Health Benefit Exchange as described in RCW 41.05.410 that meets the standard plan design and additional affordability and quality metrics required by applicable law.

1.13 ***"Standard Plan"*** means a standardized health benefit plan design developed by the Health Benefit Exchange to provide consistent cost-sharing and benefit design across all Carriers; allows consumers the ability to compare plans across Carriers.

1.14 ***"State"*** means the State of Washington, or such other state to the extent that a Coverage Agreement or Covered Person is subject to such other state's law.

2. Cascade Care Product. This Product Attachment constitutes the "Cascade Care Product (Cascade Complete Care) Attachment" and is incorporated into the Agreement. It supplements the Agreement by setting forth specific terms and conditions that apply to the Cascade Care Product with respect to which a Participating Provider has agreed to participate, and with which a Participating Provider must comply in order to maintain such participation. This Product Attachment applies with respect to the provision of health care services, supplies or

accommodations (including Covered Services) to Covered Persons enrolled in or covered by a Cascade Care Product.

3. Compliance with PO HCA Contract. Provider shall comply with any term or condition of the PO HCA Contract that is applicable to the services to be performed under the Agreement.

4. Public Option Plan Reimbursement. Provider acknowledges that the following reimbursement obligations govern the Health Plan's overall administration of the Cascade Care Product's Public Option Plan and Provider agrees that its reimbursement is subject to the obligations described herein.

4.1 Participating Provider. The total amount of reimbursement provided by Health Plan to all Public Option Participating Providers for the provision of Covered Services in the statewide aggregate to Covered Persons will not exceed 160% of the total amount that would have been reimbursed under the Medicare program for provision of same or similar services in the statewide aggregate.

4.2 Rural Provider. The reimbursement rate provided by Health Plan to Public Option Participating Providers that are certified as rural hospitals by CMS, such as critical access hospitals or sole community hospitals, for the provision of Covered Services to Covered Persons will not be less than 101% of allowable costs, as defined by CMS for purposes of Medicare cost reporting.

4.3 Primary Care Services. The reimbursement provided by Provider to a Public Option Participating Provider who is a physician with a primary specialty designation of family medicine, general internal medicine, or pediatric medicine for the provision of primary care services, as defined by HCA, to Covered Persons will not be less than 135% of the amount that would have been reimbursed under the Medicare program for provision of the same or similar services.

4.4 Compensation Schedule Override. In the event that Health Plan is obligated to modify compensation to Participating Providers in order to comply with the requirements described in this Section 4, Health Plan shall provide sixty (60) day notice of such change to the extent commercially reasonable. Provider may choose not to accept the terms of any change in compensation by giving sixty (60) day notice of intent to terminate this Product Attachment.

5. Participation. Except as otherwise provided in this Product Attachment or the Agreement, all Contracted Providers under the Agreement will participate as Participating Providers in this Cascade Care Product, and will provide to Covered Persons enrolled in or covered by a Cascade Care Product, upon the same terms and conditions contained in the Agreement, as supplemented or modified by this Product Attachment, those Covered Services that are provided by Contracted Providers pursuant to the Agreement. In providing such services, Provider shall, and shall cause Contracted Providers, to comply with and abide by the provisions of this Product Attachment and the Agreement (including the Company's policies and procedures).

5.1 Network Name. All Contracted Providers under the Agreement will participate as Participating Providers in and become part of the Cascade Complete Care network.

6. Attachments. This Product Attachment includes the Compensation Schedules for the Cascade Care Product, as indicated on Schedule B of the Agreement, each of which are incorporated herein by reference.

7. Construction. This Product Attachment supplements and forms a part of the Agreement. Except as otherwise provided herein or in the terms of the Agreement, the terms and conditions of the Agreement will remain unchanged and in full force and effect as a result of this Product Attachment. In the event of a conflict between the provisions of the Agreement and the provisions of this Product Attachment, this Product Attachment will govern with respect to health care services, supplies or accommodations (including Covered Services) rendered to Covered Persons enrolled in or covered by a Cascade Care Product. To the extent Provider or any Contracted Provider is unclear about its, his or her respective duties and obligations, Provider or the applicable Contracted Provider shall request clarification from Health Plan.

8. Term. This Product Attachment will become effective as of the Effective Date, and will be coterminous with the Agreement unless a Party terminates the participation of the Contracted Provider in this Cascade Care Product in accordance with the applicable provisions of the Agreement or this Product Attachment.

9. Confidential Information. If HCA Confidential Information provided to Health Plan under the PO HCA Contract is to be shared with Provider, then the following provisions will apply:

9.1 All Confidential Information Security Requirements (“Security Requirements”) described in “Attachment 1” to the PO HCA Contract are incorporated herein by reference in their entirety. Provider will comply with all applicable laws, regulations and Security Requirements while acquiring, accessing, using, disclosing or otherwise maintaining HCA Confidential Information.

9.2 Provider will comply with all applicable requirements of the PO HCA Contract in the event of any Breach or suspected Breach of Confidential Information. Provider agrees to indemnify and hold harmless Health Plan for any damages related to unauthorized use or disclosure of Confidential Information by Provider, its officers, directors, employees, subcontractors or agents. This obligation shall survive termination of this Product Attachment and the Agreement.

10. Debarment Certification. Provider represents and warrants that it is not presently debarred, suspended, proposed for debarment, declared ineligible, or voluntarily excluded by any State or federal department or agency from participating in transactions. Provider shall immediately notify Health Plan in writing if, during the term of the Agreement, (a) Provider becomes debarred, suspended, proposed for debarment, declared ineligible or voluntarily excluded, or (b) Provider or any of Provider’s employees are subject to disciplinary action against accreditation, certification, license and/or registration. Further, Provider shall not pay for goods and services furnished by an excluded person, at the medical direction, or on the prescription of an excluded person.

11. Work Product Rights. All data and work products produced pursuant to PO HCA Contract (collectively “Work Product”) will be considered work for hire under the U.S. Copyright Act, 17 U.S.C. §101 et seq, and will be owned by HCA. Provider will not copy or disclose, transmit or perform any Work Product or any portion thereof, in any form, to any third party.

12. Site Security. While on HCA premises, Provider will conform in all respects with physical, fire or other security policies or regulations.

13. Subcontracting and Assignment.

13.1 Provider will not enter into subcontracts for any work contemplated under the PO HCA Agreement without obtaining prior written approval of HCA. Provider will not enter into subcontracts for the provision of Covered Services to Covered Persons or any other work contemplated under this Product Attachment without obtaining prior written approval from Health Plan.

13.2 To the extent that the Agreement is considered a “Subcontract” requiring HCA approval under the PO HCA Contract, the Agreement will not take effect prior to HCA’s review and written approval.

13.3 Provider acknowledges and agrees that no assignment of the Agreement shall take effect without the prior written agreement of HCA.

14. Federal Requirements. The following requirements apply to Delegated and Downstream Entities under this Product Attachment.

14.1 Provider’s delegated activities and reporting responsibilities, if any, are specified in the Agreement or applicable attachment to the Agreement (e.g., Delegated Credentialing Agreement, Delegated

Services Agreement, Statement of Work, or other scope of services attachment) attached to this Agreement. If such attachment is not executed, no administrative functions shall be deemed as delegated.

14.2 CMS, Health Plan and Payor reserve the right to revoke the delegation activities and reporting requirements or to specify other remedies in instances where CMS, Health Plan or the Payor determine that Provider or any Downstream Entity has not performed satisfactorily.

14.3 Provider and all Downstream Entities must comply with all applicable laws and regulations relating to the standards specified under 45 CFR §156.340(a);

14.4 Provider and all Downstream Entities must permit access by the Secretary and OIG or their designees in connection with their right to evaluate through audit, inspection or other means, to the Provider's or Downstream Entities' books, contracts, computers, or any other electronic systems including medical records and documentation, relating to Health Plan's obligations in accordance with federal standards under 45 CFR §156.340(a) until ten (10) years from the termination date of this Product Attachment.

15. Termination. If at any time HCA determines that Provider is incompetent or undesirable and notifies the Health Plan of such determination, then Health Plan will immediately terminate this Product Attachment and notify Provider of the termination. Upon receiving a notice of termination, Provider must immediately cease performance of all Covered Services and other work under this Product Attachment.

16. Other Terms and Conditions. Except as modified or supplemented by this Product Attachment, the compensation hereunder for the provision of Covered Services by Contracted Providers to Covered Persons enrolled in or covered by the Cascade Care Product is subject to all of the other provisions in the Agreement (including Health Plan's policies and procedures) that affect or relate to compensation for Covered Services provided to Covered Persons.

**ATTACHMENT G: Cascade Care
Network: Cascade Complete Care**

**EXHIBIT G-2
CASCADE CARE
COMPENSATION SCHEDULE
PRACTITIONER SERVICES**

RadiantCare Physicians PLLC

This Compensation Schedule sets forth the maximum reimbursement amounts for the provision of Covered Services to Covered Persons in a Cascade Care Product offered through Health Plan. For Covered Services rendered to a Covered Person and billed under a Contracted Provider's tax identification number ("TIN") that has been designated by the Payor as subject to this Compensation Schedule, Payor shall pay or arrange for payment of a Clean Claim for Covered Services rendered by the Contracted Provider according to the terms of the Agreement and this Compensation Schedule. Payment under this Compensation Schedule is subject to the requirements set forth in the Agreement, which include reducing the Allowed Amount by the applicable Cost-Sharing Amounts and the Public Option Plan Reimbursement obligations described in the Cascade Care Product Attachment.

For Practitioner Covered Services provided to Covered Persons, Contracted Provider's maximum compensation shall be the Allowed Amount. Except as otherwise provided in this Compensation Schedule, the Allowed Amount is the lesser of: (i) the Contracted Provider's Allowable Charges; or (ii) one hundred percent (100%) of the Medicare Fee Schedule in effect on the date of service.

For Primary Care Practitioner Covered Services provided to Covered Persons, Contracted Provider's maximum compensation shall be the Allowed Amount. Except as otherwise provided in this Compensation Schedule, the Allowed Amount is the lesser of: (i) the Contracted Provider's Allowable Charges; or (ii) one hundred thirty-five percent (135%) of the Medicare Fee Schedule in effect on the date of service.

Multiple Procedure Pricing Rules. Multiple procedures performed during the same day will be reimbursed at one hundred percent (100%) for the primary procedure, fifty percent (50%) for the second procedure, and fifty percent (50%) for the third procedure, subsequent procedures shall not be eligible for reimbursement.

Additional Provisions:

1. **Code Change Updates.** Payor utilizes nationally recognized coding structures (including, without limitation, revenue codes, CPT codes, HCPCS codes, ICD codes, national drug codes, ASA relative values, etc., or their successors) for basic coding and descriptions of the services rendered. Updates to billing-related codes shall become effective on the date ("Code Change Effective Date") that is the later of: (i) the first day of the month following sixty (60) days after publication by the governmental agency having authority over the applicable product of such governmental agency's acceptance of such code updates, (ii) the effective date of such code updates as determined by such governmental agency or (iii) if a date is not established by such governmental agency or the product is not regulated by such governmental agency, the date that changes are made to nationally recognized codes. Such updates may include changes to service groupings. Claims processed prior to the Code Change Effective Date shall not be reprocessed to reflect any such code updates.
2. **Fee Change Updates.** Updates to the fee schedule shall become effective on the effective date of such fee schedule updates, as determined by the Payor ("Fee Change Effective Date"). However, the date of implementation of any fee schedule updates, i.e. the date beginning on which such fee change is used for reimbursement ("Fee Change Implementation Date") shall be the later of: (i) the date on which Payor is reasonably able to implement the update in the claims payment system; or (ii) the Fee Change Effective Date. Claims processed prior to the Fee Change Implementation Date shall not be reprocessed to reflect any updates to such fee schedule, even if service was provided after the Code Change Effective Date.

3. Modifier. Unless specifically indicated otherwise, the Allowed Amount represents global fees and may be subject to reductions based on appropriate modifiers (for example, professional and technical modifiers). As used in the previous sentence, “global fees” refers to services billed without a modifier, for which the Allowed Amount includes both the professional component and the technical component.
4. Anesthesia Modifier Pricing Rules. The dollar amounts that will be used in the calculation of Anesthesia Management fees are in accordance with the Anesthesia Payment Policy. Unless specifically stated otherwise, the Anesthesia Conversion Factor indicated is fixed and will not change. The Anesthesia Conversion Factor is based on an anesthesia time unit value of 15 minutes.
5. Place of Service Pricing Rules. Payor will follow CMS guidelines for determining when services are priced at the facility or non-facility fee schedule (with the exception of services performed at Ambulatory Surgery Centers, POS 24, which will be priced at the facility fee schedule).

Definitions:

1. **Allowed Amount** means the amount designated as the maximum amount payable to a Contracted Provider for any particular Covered Service provided to any particular Covered Person, pursuant to this Agreement or its Attachments for Covered Services.
2. **Allowable Charges** means a Contracted Provider’s billed charges for services that qualify as Covered Services.
3. **Cost-Sharing Amounts** means any amounts payable by a Covered Person, such as copayments, cost-sharing, coinsurance, deductibles or other amounts that are the Covered Person’s financial responsibility under the applicable Coverage Agreement, if applicable.